

The Heavy Little Things

A CPTSD Toolkit for Women: Healing
Complex Trauma, Calming the
Nervous System, and Finding Safety
in Yourself

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For every woman carrying the weight of a childhood
she did not choose —
you are not your wounds.
You are the incredible person who survived them.

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Foreword: The Weight You Carry

THERE ARE THE BIG THINGS — THE OBVIOUS TRAUMAS THAT LEAVE MARKS EVERYONE CAN SEE. AND THEN THERE ARE THE HEAVY LITTLE THINGS. THE MOMENTS THAT SEEMED SMALL AT THE TIME BUT ACCUMULATED UNTIL THEY BECAME A WEIGHT YOU'VE CARRIED SO LONG YOU FORGOT IT WAS THERE. THE MICRO-REJECTIONS. THE TIMES YOU WEREN'T PROTECTED WHEN YOU SHOULD HAVE BEEN. THE THINGS YOU HAD TO BE FOR OTHER PEOPLE BEFORE YOU LEARNED WHO YOU WERE.

If you picked up this book, you probably know what I mean. You carry a weight that doesn't have a single name. You react to things in ways that seem out of proportion. You're exhausted in a way that sleep doesn't fix. You've been told you're "too sensitive" or "dramatic" or that you "need to let things go" — as if letting go was a switch you could flip.

This book is about that weight. It's about complex trauma — CPTSD — which is what happens when trauma isn't a single event but a sustained environment. When the threat wasn't a moment but a childhood, a relationship, a system.

Why This Book Exists

There are excellent books about trauma. Judith Herman, Bessel van der Kolk, Pete Walker, Janina Fisher — their work changed the field. But most of them are either clinical textbooks or general guides. There are very few practical toolkits that address CPTSD specifically through a women's lens — and that's the gap this book fills.

Women are disproportionately affected by complex trauma. We are more likely to experience childhood abuse, intimate partner violence, sexual trauma, emotional neglect, and the chronic stress of living in a world that devalues us. And we are also more likely to be told our trauma responses are "personality flaws" rather than survival strategies.

This book is a toolkit, not a theory book. Every chapter ends with concrete, actionable strategies. This is not about "healing" in some abstract, forever sense — it's about practical steps for calming your nervous system, taming your inner critic, setting boundaries, and building a life that feels safe.

How to Use This Book

The chapters follow a logical arc: understanding CPTSD, working with your inner world, regulating your nervous system, healing relationships, building safety. But you don't have to read them in order. If emotional

flashbacks are your biggest struggle, start at Chapter 3. If you can't set boundaries to save your life, start at Chapter 6. If the inner critic is running your show, start at Chapter 7.

Each chapter ends with a Toolkit Summary — real strategies, not theory.

One Last Thing

Healing is not linear. You will take two steps forward and one step back. You will think you've dealt with something only to have it surface again. That's not failure — that's how complex trauma works. It had years to embed itself. It takes years to unwind.

Be patient with yourself. The weight didn't appear overnight, and it won't disappear overnight. But it can get lighter. One heavy little thing at a time.

Toolkit Summary

- **Name one heavy little thing.** Just identifying one pattern — one trigger, one belief, one response — reduces its power.
- **Stop trying to "get over it."** CPTSD doesn't work that way. You're not failing at healing — you're healing from something complex.
- **Pick one chapter that feels urgent.** Read it. Try one strategy.

- **Healing happens in safety.** If you're not safe right now (in your home, your relationship, your life), focus on safety first. The rest can wait.

Chapter 1: Understanding Complex Trauma

YOU'VE BEEN CARRYING SOMETHING HEAVY FOR A LONG TIME. MAYBE YOU DON'T EVEN REMEMBER WHEN YOU FIRST PICKED IT UP. IT'S JUST ALWAYS BEEN THERE — A WEIGHT IN YOUR CHEST, A TIGHTNESS IN YOUR SHOULDERS, A VOICE THAT TELLS YOU YOU'RE NOT ENOUGH, NOT SAFE, NOT OKAY. YOU'VE LEARNED TO MOVE THROUGH LIFE WITH THIS WEIGHT. YOU'VE BUILT ROUTINES, RELATIONSHIPS, AND A WHOLE LIFE AROUND IT. BUT SOME DAYS, THE WEIGHT FEELS UNBEARABLE. SOME DAYS, YOU WONDER WHY EVERYTHING SEEMS HARDER FOR YOU THAN FOR OTHER PEOPLE.

That weight has a name. It's called complex trauma.

CPTSD vs. PTSD: More Than a Single Event

Most people have heard of PTSD — post-traumatic stress disorder. We think of it as something that happens after a single, identifiable event: a car accident, an assault, a natural disaster. The symptoms make sense in

that context. You experience something terrifying, and your nervous system stays on high alert afterward, ready to protect you from it happening again.

Complex PTSD, or CPTSD, is different. It doesn't come from one event. It comes from prolonged, repeated exposure to trauma — especially during childhood or in relationships where you couldn't escape. It's the accumulation of thousands of small weights, added one by one, day after day, year after year. Emotional neglect. Chronic criticism. Unpredictable parenting. Manipulation. Control. Gaslighting. Physical or emotional demands that no child should have to meet.

Where PTSD is like a single large rock dropped onto your path, CPTSD is like being forced to carry a backpack that someone keeps adding stones to, silently, every single day, for years. You don't notice each individual stone. But eventually, the weight shapes everything about how you move through the world. It changes your posture, your expectations, your sense of what is normal. You don't know what it would feel like to walk without it.

The Diagnostic Criteria

The World Health Organization recognized CPTSD as a distinct diagnosis in the ICD-11 in 2018. This was a significant moment — an official acknowledgment that prolonged, inescapable trauma creates a different kind of wound than single-incident trauma. It includes all the core symptoms of PTSD — re-experiencing, avoidance,

and a heightened sense of threat — plus three additional clusters of symptoms that arise specifically from prolonged, inescapable trauma.

Disturbances in self-organization. This means your sense of who you are is fractured or unclear. You might feel fundamentally different from other people, as though you're defective or broken. You might not know what you want, what you believe, or who you are without the trauma defining you. Your identity feels unstable, borrowed from whoever is around you, because you never had the safety to develop one of your own.

Difficulties with relationships. After years of unsafe relationships, your nervous system has learned that people are dangerous. You might swing between desperately wanting connection and pushing people away. Trust feels impossible, or you give it too quickly to the wrong people. Relationships feel exhausting because you're always scanning for danger, always anticipating abandonment, always trying to be what the other person needs.

Problems with emotional regulation. Emotions feel overwhelming. You might have outbursts of anger that feel outsized to the situation, or you might numb out altogether. Sadness can feel bottomless. Fear can feel like it's everywhere, even when there's no obvious threat. You might feel like your emotions control you rather than the other way around.

If you're reading this and recognizing yourself, please know: this is not a personality flaw. This is not you being "too sensitive" or "too much." This is your nervous system doing exactly what it evolved to do — protecting you from harm, even long after the original danger has passed. The symptoms are not evidence of brokenness. They are evidence of adaptation to an environment that should never have been yours to endure.

The Nervous System: A Polyvagal View in Plain Language

Stephen Porges's polyvagal theory offers a framework for understanding what's happening inside you. It sounds complicated, but the core idea is simple, and it changes everything once you understand it.

Your nervous system has three main states, arranged like a ladder. You are always somewhere on this ladder, shifting between states based on what your system perceives as safe or dangerous.

The Ventral Vagal State (Safety and Connection). This is your home base. When you feel safe, your body relaxes. Your breathing is easy and full. You can make eye contact, laugh with a friend, and engage with the world. Your heart rate is steady, your digestion works properly, and you feel a sense of social connection. In this state, you can think clearly, feel your emotions without being

drowned by them, and reach out to others. This is the state where healing happens. This is the state where you can truly rest.

The Sympathetic State (Fight or Flight). When your nervous system detects danger, it activates your sympathetic nervous system. Your heart rate increases. Your breathing becomes shallow and rapid. Your muscles tense. Your pupils dilate. You're ready to fight or run. Blood moves away from your digestive system and into your large muscles. This state is useful in real danger — it's what saved your ancestors from predators. But when you have CPTSD, your system can get stuck here. You feel on edge, irritable, anxious, and hypervigilant — even when you're perfectly safe. Your body is ready for a threat that no longer exists.

The Dorsal Vagal State (Freeze or Shutdown). When the danger is overwhelming and you can't fight or flee, your nervous system pulls the emergency brake. You dissociate. Your body goes numb. Your voice gets small or disappears. You feel disconnected from yourself and the world around you. Your blood pressure drops. Your heart rate slows. You might feel exhausted, empty, or like you're watching your life from a great distance. This is the state of collapse. It's your system's last-resort survival strategy.

For people with CPTSD, the nervous system gets stuck moving between the sympathetic state and the dorsal vagal state — between chronic hyperarousal and shutdown. The ventral vagal state — true safety and

connection — becomes hard to access. Your system has forgotten what safety feels like. It no longer has a template for calm.

The good news is that the nervous system is plastic. It can learn new patterns. Safety can be relearned, one experience at a time. You can teach your system, gently and repeatedly, that you are safe now.

Why Women Are Disproportionately Affected

This is not a contest. Trauma is trauma, and it affects people of all genders. But complex trauma does have a particular weight for women, and it's worth understanding why — not to compare suffering, but to validate what so many women experience in silence.

Women are statistically more likely to experience certain forms of prolonged, relational trauma — childhood emotional abuse, sexual abuse, intimate partner violence, and what researchers call "betrayal trauma," where the people who should have protected you were the ones who hurt you. Women are also more likely to experience trauma within relationships that are supposed to be safe — family, intimate partnerships, caregiving relationships — which creates a uniquely confusing and painful wound.

Add to this the cultural conditioning that teaches women to put others' needs before their own, to stay quiet, to be "good" and "nice" and compliant. Girl children

are often expected to manage adult emotions, to absorb criticism without complaint, to be small and accommodating. When you are trained from childhood to shrink yourself, the trauma goes unnoticed. It becomes part of the background of your life. You don't realize how much weight you're carrying because you've never known anything different.

Many women with CPTSD have spent their whole lives being told they're "too emotional" or "too dramatic" or "overreacting" — when in reality, their nervous systems are responding normally to an abnormal history. The weight you carry is real. And you didn't deserve what put it there.

The Window of Tolerance

Imagine a window. When you're inside this window, you can think clearly, feel your emotions without being overwhelmed by them, and stay present in your body. This is the zone of optimal arousal. You can handle life's ups and downs because you have enough capacity to process them. Stress comes, you feel it, and you recover.

When you have CPTSD, your window of tolerance gets narrower. Your nervous system's threshold for activation is lower, and its recovery time is longer. Small stressors that another person might brush off can push you out of the window — into hyperarousal (panic, anger, racing thoughts, restlessness) or into hypoarousal (numbness, dissociation, collapse, exhaustion). You are not reacting to

the present stressor alone. You are reacting to the present stressor plus the accumulated weight of every unprocessed stressor from your past.

Here's the important part: your window of tolerance is not fixed. It is not a permanent limitation. It can expand. With the right tools, consistent practice, and compassionate support, you can widen that window. What overwhelms you today might be manageable a year from now. Your capacity can grow. The nervous system learns through experience, and every time you successfully navigate a difficult moment without going into survival mode, you are teaching your system that it can handle more than it thought possible.

The Myth of "Getting Over It"

If one more person tells you to "just let it go" or "move on" or "stop living in the past" or "forgive and forget," you have my full permission to roll your eyes. This advice is rooted in a fundamental misunderstanding of what complex trauma actually is.

Complex trauma is not a bad mood you can shake off. It's not a grudge you're choosing to hold. It's not a memory you could simply decide to stop thinking about. It is a nervous system that has been shaped by years — sometimes decades — of threat. It is a body that learned to expect danger before it learned to expect safety. It is a set of survival adaptations that were brilliant when you needed them and are now stuck in the on position.

You can't think your way out of a nervous system that doesn't feel safe. You can't reason with a body that has learned the world is dangerous. Healing is not about "getting over" anything. It's about slowly, gently teaching your system that safety is possible now. It's about building trust with yourself, one small experience at a time.

Healing is about lightening the load, one small piece at a time. It's about recognizing that the weight was never supposed to be yours to carry alone. It's about learning to set down what was never yours to hold.

This book is not about fixing you, because you were never broken. You are a person who has been carrying an unfair weight, and this book is about learning how to set some of it down.

That is not the same as getting over it. It is far more compassionate than that.

As you move through this book, you will find practical tools and strategies — not just concepts to understand, but things you can actually do. Some will resonate. Some won't. Take what fits, leave what doesn't. Healing is not about doing everything right. It's about finding what helps you set down the weight, even if only a little bit at a time.

Toolkit Summary

- CPTSD differs from PTSD — it comes from prolonged, repeated trauma rather than a single event
- The three core symptom clusters beyond PTSD: disturbances in self-organization, relationship difficulties, and emotional regulation problems
- The nervous system has three states: ventral vagal (safety and connection), sympathetic (fight/flight), and dorsal vagal (freeze/shutdown)
- Women are disproportionately affected due to higher rates of relational trauma and cultural conditioning to prioritize others
- The window of tolerance describes your capacity to process stress — and it can expand with practice
- Healing is not about "getting over" trauma; it's about teaching your nervous system that safety is possible
- Your weight is real, and you were never broken — you are a person learning to set down what was never yours to carry

Chapter 2: The Wounded Inner Self

IF YOU HAVE CPTSD, YOU'VE PROBABLY NOTICED THAT YOU DON'T FEEL LIKE ONE CONSISTENT PERSON. SOME DAYS YOU'RE CAPABLE AND CONFIDENT. OTHER DAYS YOU'RE FLOODED WITH SELF-CRITICISM SO LOUD YOU CAN'T THINK. SOMETIMES YOU'RE THE ONE TAKING CARE OF EVERYONE ELSE. SOMETIMES YOU'RE A TERRIFIED CHILD WHO JUST WANTS SOMEONE TO MAKE IT STOP. SOMETIMES YOU FEEL LIKE NO ONE AT ALL — JUST A HOLLOW SHELL GOING THROUGH THE MOTIONS.

This isn't fragmentation in the scary, pathological sense. This is something far more common and far more human. You are not one single voice inside your own mind. You are a whole inner world — a cast of characters, each with their own feelings, memories, jobs, and fears. This is what trauma experts call "parts work," and it's one of the most gentle and effective ways to understand what's happening inside you.

Parts Work for CPTSD

The Internal Family Systems (IFS) model, developed by Dr. Richard Schwartz, offers a compassionate framework for understanding these inner parts. The name sounds clinical, but the idea is beautifully simple: your mind is

not a unified self. It's a system of sub-personalities, or "parts," that developed to help you survive. At the center of this system is what IFS calls the "Self" — your core, undamaged essence, characterized by qualities like compassion, curiosity, calm, courage, and creativity.

When you grew up in an environment that wasn't consistently safe, your parts had to work overtime. They took on extreme roles because the circumstances demanded extreme solutions. A part of you learned to be hypervigilant to other people's moods, scanning for the slightest sign of danger. Another part learned to shut down your own needs, because needing things was unsafe. Another part learned to criticize you so harshly that you'd stay small and invisible — because being invisible was safer than being seen.

These parts aren't your enemies. They are not pathologies to be eliminated. They are protectors and survivors who took on impossible jobs when you were too young and too vulnerable to manage on your own. They are like firefighters who showed up to the fire, even if they sometimes broke a few windows getting in. Their methods may not be ideal, but their intention is always protection.

The Inner Critic

The inner critic is likely the part you know best. It's the voice inside your head that tells you you're not good enough, that you messed up again, that everyone can see

how flawed you are. It reviews every social interaction for mistakes. It compares you to others and finds you lacking. It demands perfection as the price of being acceptable. It whispers: "If you had just tried harder. If you had just been better. If you hadn't said that thing. Maybe then they would have loved you."

Here's what the inner critic actually is: a protector. A part of you that learned that if it could criticize you into being perfect, maybe you would be safe from the criticism, punishment, or neglect you experienced growing up. If you could be flawless, maybe people wouldn't hurt you. Maybe they'd finally love you.

The inner critic is not an abuser living inside your head. It is a scared young part that picked up a brutal job. It genuinely believes it's helping you. It thinks that if it stops working, you'll let your guard down and disaster will follow. And when you start to heal, it often gets louder at first — because it's terrified that if it stops doing its job, you'll be in danger. The critic's escalation is not a sign that healing isn't working. It's a sign that the critic is scared and needs reassurance.

The goal isn't to silence the inner critic. The goal is to thank it for its service and gently show it that you're safe now. That you don't need it to work so hard anymore. That you've got this.

The Exile

Underneath the inner critic, underneath all the protective parts, live the exiles. These are the youngest parts of you — the ones who hold the raw emotions from traumatic experiences. Shame. Terror. Grief. Loneliness. Abandonment. Rage that was never allowed to be expressed. These feelings were too overwhelming to process at the time, so parts of you got locked away, frozen in the moment they were hurt.

Exiles are called exiles because they've been pushed out of conscious awareness. You don't want to feel that shame. You don't want to touch that grief. So the protective parts keep the exiles hidden — sometimes so effectively that you don't even know they're there. The protectors build walls around the exiles to keep their pain from flooding your system.

But exiles don't stay hidden forever. They get triggered. You meet someone who reminds you of a parent, and suddenly you're flooded with a feeling that makes no sense in the present moment. You hear a certain tone of voice, and you're six years old again, desperate and small. A particular smell, a season, a song — and suddenly the exile is right there, all that frozen pain thawing at once.

The exile is the younger you who is still waiting. Waiting to be heard. Waiting to be comforted. Waiting for someone to finally say, "That shouldn't have happened to you. You didn't deserve that. You are loved." The exile

doesn't need to be fixed or erased. The exile needs to be witnessed. It needs someone to finally see what happened and to stay.

The Protector

The protector is the part that keeps you armored. It's the part that might keep people at a distance, that doesn't let anyone get too close. It might be the part that stays busy all the time, so you don't have to feel. It might be the part that numbs out with food, alcohol, work, social media, or endless television. It might be the part that intellectualizes everything, staying safely in your head so you never have to drop into your body.

Protectors are not bad. They are brilliant strategies that kept you alive. They shielded you when you had no other shield. They gave you a way to function in a world that felt fundamentally unsafe. But protectors also don't know that the danger has passed. They're still operating on old information. They don't realize you're an adult now, with resources and choices you didn't have before. They think they're still protecting a child.

When you try to heal, protectors often push back hard. They see healing as dangerous. "If we let our guard down," they say, "we'll be hurt again." They might resist therapy, sabotage your self-care routines, or flood you with doubt. This resistance is not a sign that you're doing something wrong. It's a sign that your protector parts are

strong and loyal. They need to be treated with respect, not pushed aside. They need to be invited into healing rather than overruled by it.

The People-Pleaser

The people-pleaser is a specific kind of protector that deserves its own name. This is the part that learned early that keeping everyone else happy was the surest path to safety. If you could anticipate their needs, meet their expectations, and never cause any trouble, maybe they wouldn't get angry. Maybe they wouldn't leave. Maybe they would finally be pleased with you.

The people-pleaser is not weak. It is incredibly perceptive and resourceful. It learned to read micro-expressions, tone shifts, and silences with remarkable precision. It developed a sophisticated strategy for survival in a world where your safety depended on someone else's mood. It knows how to monitor a room, how to deflect tension with a joke, how to become invisible or charming as the situation demands. That is not weakness; that is intelligence born of necessity.

But the weight of always putting others first takes a toll. The people-pleaser is exhausted. It has been working without rest for years, maybe decades. It needs to know that it can rest now. That you are allowed to have needs. That saying no is safe. That people will not abandon you simply because you are not constantly serving them.

Befriending Your Parts Instead of Fighting Them

For years, you may have tried to fight your inner critic, to banish your fear, to push away your sadness. This is understandable. These parts feel like problems to be solved, enemies to be defeated. But fighting your parts only makes them work harder. It's like yelling at a frightened child to calm down — it doesn't work, and it makes everything worse.

Befriending your parts is a different approach entirely. It starts with curiosity instead of judgment. When the inner critic starts shouting, instead of fighting back, you can pause and ask: "Hello, critic. I see you're here. What are you afraid would happen if you stopped?" When fear grips you, you can ask: "What are you trying to protect me from right now?"

This practice of getting curious rather than reactive creates space. It separates you from the part. In IFS terms, it helps you access "Self-energy" — that calm, compassionate center that can hold all your parts with love. When you're in Self-energy, you can listen to your parts without being consumed by them. You can be the calm adult in the room of your own mind.

This is not about getting rid of your parts. They are not tumors to be excised. Your parts can transform. When they are freed from their extreme roles and allowed to rest, they reveal their true nature — and their true nature is often beautiful. The inner critic, when it's not working

so hard, might become your sharpest discernment. The protector might become your grounded boundary. The people-pleaser might become your genuine kindness.

The Self: Who You Are Underneath the Parts

IFS teaches that underneath all your protective and exiled parts, there is a Self. This Self is not a part. It is who you are when none of your parts are running the show. It has eight qualities, often called the "8 C's": calm, curiosity, compassion, confidence, courage, clarity, creativity, and connectedness.

You may not feel like you have access to this Self right now. The parts are too loud, the protectors too active. But the Self is never damaged by trauma. It is simply obscured — covered over by the extreme roles your parts had to take on. As you befriend your parts and help them relax, more of your Self naturally emerges. You don't have to build it. It was always there.

The Younger You Who Is Still Waiting

There is a version of you who is still in that childhood room, still waiting for someone to come and make it better. That younger you exists inside your nervous system, holding the memories, the feelings, the unmet

needs. She may be very young. She may be wearing the same clothes, feeling the same dread, hoping for the same rescue that never came.

You are the only one who can go back for her. Not to rescue her from the past — what happened already happened, and that is a grief that deserves to be held. But to sit with her. To let her know she is not alone anymore. To tell her what she needed to hear then and still needs to hear now: "I see you. I hear you. You matter. And I will take care of us now."

This is not about erasing the past. It is not about pretending the hurt didn't happen. It is about bringing the younger you into the present, where she can finally be held by the adult she grew up to become. Where the weight she's been carrying can finally be shared. Where she can rest.

Because you are no longer a child, and you are no longer helpless. You are an adult with capacity, resources, and the willingness to heal. And that is enough. You have always been enough.

Toolkit Summary

- Your mind is a system of parts, not a single unified voice — and at the center is your undamaged Self

- The inner critic is a protector that learned to keep you safe through self-criticism and perfectionism; thank it, don't fight it
- Exiles hold the raw pain of traumatic experiences, frozen in time — they need to be witnessed, not fixed
- Protectors developed strategies to shield you from harm and still need respect, not force
- The people-pleaser learned that keeping others happy was the safest strategy — it is intelligent and exhausted
- Befriending your parts means approaching them with curiosity and compassion instead of combat
- The Self (calm, curious, compassionate) is never damaged by trauma — only obscured by protective parts
- The younger you is still waiting to be heard — and you are the one who can finally show up for her
- Your parts are not problems; they are survivors who deserve your compassion, not your rejection

Chapter 3: Emotional Flashbacks and Regulation

YOU'RE SITTING AT A DINNER TABLE WITH PEOPLE YOU LOVE. EVERYTHING IS FINE — OBJECTIVELY FINE. NO ONE IS ANGRY. NO ONE IS IN DANGER. AND THEN SOMEONE MAKES A COMMENT, OR THE TONE OF SOMEONE'S VOICE SHIFTS, OR THE LIGHT HITS THE ROOM IN A CERTAIN WAY, AND SUDDENLY YOU ARE DROWNING. YOUR CHEST TIGHTENS. YOUR THROAT CLOSES. A WAVE OF SHAME OR TERROR CRASHES OVER YOU, AND YOU HAVE NO IDEA WHY.

Your rational mind says: nothing is wrong. But your body is screaming that something is very, very wrong. You feel small, helpless, terrified. You want to disappear. You don't know what hit you or why you can't make it stop.

This is an emotional flashback.

What Emotional Flashbacks Are (and Are Not)

When we hear the word "flashback," most of us think of a movie version — vivid visual memories replaying like a film. A soldier hearing a helicopter and being back in combat. A survivor seeing the face of their attacker. But

for complex trauma, flashbacks rarely look like that. They don't come with images. They don't come with a clear narrative. They come as pure, overwhelming feeling.

An emotional flashback is a somatic and emotional re-experiencing of a traumatic time, without the conscious memory of what happened. Your body and nervous system are suddenly transported back to a childhood state — helpless, small, ashamed, terrified — even though your adult mind can see that nothing is wrong in the present moment. The feeling is the memory. The body remembers what the mind has protected you from knowing in detail.

This is disorienting. It's frightening. And it's one of the most common and painful symptoms of CPTSD. In an instant, you shift from being a capable adult to being the child who was never safe. You lose access to your adult perspective. You become flooded with feelings that don't seem to belong to now but feel completely real.

The difference between a flashback and a feeling is this: feelings are responses to the present moment. They have a clear trigger in the here and now. You feel sad because you received bad news. You feel angry because someone wronged you. A flashback, in contrast, is a response to the past that feels like the present. The trigger might be present — a tone of voice, a facial expression, a certain smell — but the intensity of the response does not match the trigger. When you're angry about something that just happened, that's a feeling. When you're filled with a sense of existential terror,

shame, or worthlessness that has no proportional relationship to what's actually in front of you, that's likely an emotional flashback.

The 13 Steps for Flashbacks (Adapted from Pete Walker)

Pete Walker, author of *Complex PTSD: From Surviving to Thriving*, developed a set of steps for navigating emotional flashbacks. These steps are not a rigid protocol — they are tools you can adapt. Over time, they become second nature. Here they are, adapted and expanded:

1. **SAY IT OUT LOUD: "I'M HAVING A FLASHBACK."** THE MOMENT YOU NAME WHAT'S HAPPENING, YOU CREATE DISTANCE BETWEEN YOUR ADULT SELF AND THE YOUNGER SELF WHO IS RELIVING THE PAST. THE FLASHBACK OPERATES IN SECRECY AND CONFUSION. SPEAKING ITS NAME DIMINISHES ITS POWER. JUST SAYING THE WORDS HELPS YOUR PREFRONTAL CORTEX COME BACK ONLINE.
2. **REMIND YOURSELF: "I AM SAFE NOW."** THIS IS NOT TOXIC POSITIVITY. IT'S A FACTUAL STATEMENT YOU REPEAT TO YOUR NERVOUS SYSTEM. "I AM AN ADULT. THE DANGER IS IN THE PAST. I AM IN [CURRENT YEAR]. I AM IN [CURRENT LOCATION]. LOOKING AROUND THE ROOM, I CAN SEE THAT NO ONE IS THREATENING ME. I AM SAFE RIGHT NOW."

3. **OWN YOUR RIGHT TO BE HERE.** YOU HAVE A RIGHT TO OCCUPY SPACE. YOU HAVE A RIGHT TO EXIST, TO BREATHE, TO TAKE UP ROOM ON THIS EARTH. THE FLASHBACK MAY TELL YOU THAT YOU'RE IN DANGER OF BEING ERASED OR ANNIHILATED, BUT THAT IS THE PAST SPEAKING. PLANT YOUR FEET ON THE GROUND AND CLAIM YOUR SPACE.
4. **GROUND YOURSELF IN YOUR BODY.** PRESS YOUR FEET INTO THE FLOOR. SQUEEZE YOUR HANDS TOGETHER. FEEL THE WEIGHT OF YOUR BODY IN THE CHAIR. TOUCH SOMETHING TEXTURED — A WALL, A BLANKET, A PIECE OF FABRIC. LET YOUR SENSES ANCHOR YOU TO THE PRESENT. NOTICE THE TEMPERATURE OF THE AIR, THE LIGHT IN THE ROOM, THE SOUNDS AROUND YOU.
5. **BREATHE SLOWLY AND GENTLY.** INHALE FOR FOUR COUNTS. HOLD FOR FOUR. EXHALE FOR SIX. THE LONG EXHALE TELLS YOUR VAGUS NERVE THAT IT'S SAFE TO COME OUT OF THREAT RESPONSE. BREATHE LIKE YOU'RE CALMING A FRIGHTENED ANIMAL — BECAUSE YOU ARE. YOUR NERVOUS SYSTEM IS THAT ANIMAL.
6. **FIND YOUR INNER PROTECTOR.** REMEMBER THAT YOU ARE AN ADULT NOW WITH ADULT RESOURCES. YOU HAVE A DRIVER'S LICENSE, A BANK ACCOUNT, A PHONE, THE ABILITY TO CALL FOR HELP, THE ABILITY TO LEAVE A SITUATION. YOU CAN PROTECT THE YOUNGER YOU

WHO IS FLOODING THE PRESENT. VISUALIZE YOUR ADULT SELF STANDING BETWEEN THE YOUNGER YOU AND WHATEVER THREATENED HER.

7. **GRIEVE THE FLASHBACK.** THE FLASHBACK IS HAPPENING BECAUSE SOMETHING TERRIBLE HAPPENED TO YOU. YOU DESERVED BETTER. YOU DESERVED SAFETY AND LOVE AND PROTECTION. LET YOURSELF FEEL THE SADNESS OF THAT. TEARS ARE REGULATION IN ACTION. THEY WASH STRESS HORMONES OUT OF YOUR SYSTEM AND SIGNAL TO YOUR BODY THAT THE THREAT IS PASSING.
8. **USE SAFE TOUCH.** PLACE A HAND ON YOUR HEART. GENTLY RUB YOUR OWN ARM. WRAP YOUR ARMS AROUND YOURSELF IN A GENTLE HUG. THESE GESTURES SIGNAL SAFETY TO YOUR NERVOUS SYSTEM IN A LANGUAGE IT UNDERSTANDS — THE LANGUAGE OF PHYSICAL COMFORT.
9. **PREVENT FLASHBACK DRIFT INTO SHAME.** THE FLASHBACK WILL TRY TO CONVINCE YOU THAT YOU'RE BAD, DEFECTIVE, OR UNLOVABLE. IT WILL TRY TO TURN THE EXPERIENCE INTO EVIDENCE AGAINST YOU. RECOGNIZE THIS AS THE FLASHBACK TALKING, NOT TRUTH. SAY TO YOURSELF: "THAT THOUGHT IS THE FLASHBACK TALKING. IT IS NOT REALITY. I AM NOT DEFINED BY THIS FLASHBACK."
10. **BE PATIENT.** FLASHBACKS RARELY END INSTANTLY. THEY HAVE AN ARC. THEY BUILD, THEY PEAK, AND THEY SUBSIDE. YOUR JOB IS NOT TO FORCE THEM AWAY

OR SHUT THEM DOWN. YOUR JOB IS TO STAY PRESENT THROUGH THEM — TO HOLD SPACE FOR THE FEELING WITHOUT BECOMING IT.

11. **RETURN TO YOUR BODY.** ONCE THE INTENSITY STARTS TO PASS, BRING YOUR ATTENTION GENTLY BACK TO YOUR PHYSICAL BODY. NOTICE YOUR BREATH. NOTICE THE CONTACT OF AIR ON YOUR SKIN. NOTICE THE SUPPORT OF THE SURFACE BENEATH YOU. NOTICE THAT YOU MADE IT THROUGH.
12. **USE A "FLASHBACK-STOPPER" PHRASE.** SAY TO YOURSELF: "I'M AN ADULT NOW. I HAVE RESOURCES. I HAVE CHOICES. I AM NOT HELPLESS." REPEAT IT AS MANY TIMES AS YOU NEED.
13. **PRACTICE SELF-COMPASSION AFTER.** AFTER A FLASHBACK, THE INNER CRITIC OFTEN RUSHES IN TO JUDGE YOU FOR HAVING IT. "YOU'RE STILL NOT OVER THIS? IT'S BEEN YEARS. YOU SHOULD BE BETTER BY NOW." MEET THAT JUDGMENT WITH KINDNESS. YOU SURVIVED SOMETHING HARD. THE FLASHBACK IS EVIDENCE OF YOUR NERVOUS SYSTEM TRYING TO HEAL, NOT EVIDENCE OF FAILURE.

Grounding Techniques for Women

Grounding is the art of bringing yourself back to the present moment through your senses. When you're in a flashback, your brain is living in the past. Grounding pulls it gently into the now.

The 5-4-3-2-1 technique. Name five things you can see. Four things you can touch. Three things you can hear. Two things you can smell. One thing you can taste. This forces your brain to engage with the present sensory environment, pulling attention away from the internal flashback state.

Temperature grounding. Hold an ice cube. Splash cold water on your face. Step outside into cool air. Take a warm shower. Temperature shifts can interrupt a flashback more quickly than almost any other technique because they send a strong, undeniable signal to your nervous system.

Object grounding. Keep a small grounding object in your pocket — a smooth stone, a textured keychain, a piece of fabric, a bracelet with beads. When a flashback hits, hold it and describe it in detail to yourself. "This stone is cool and smooth. It has a small crack on the left side. It fits perfectly in my palm. The light reflects off this one spot."

Movement grounding. Stand up and shake out your hands and feet. Jump gently. Press your palms together firmly. Stretch your arms overhead. Movement sends proprioceptive signals to your brain that say: "I am here, in this body, in this moment."

Voice grounding. Hum. Sing a few notes. Make a sound. The vagus nerve runs through the vocal cords, and using your voice activates it, promoting calm.

Regulation Skills: The Glimmer Practice, Orienting, and Pendulation

Grounding helps you in the middle of a flashback. Regulation helps you build a nervous system that has fewer and less intense flashbacks over time. These are daily practices for nervous system health, not just emergency tools.

The glimmer practice. Deb Dana, a clinician who applies polyvagal theory, uses the term "glimmer" to describe the small moments of safety and connection that our nervous system often overlooks but desperately needs. A glimmer can be the warmth of sunlight on your skin, the sound of a bird outside your window, the feeling of a warm mug in your hands, the sight of a tree swaying in the wind, the weight of a cat curled up on your lap.

The practice is simple: actively look for glimmers throughout your day. Each time you notice one, pause for five full seconds and let your nervous system register it. Feel it in your body. Let the sensation of safety seep in. You are literally training your system to recognize safety again. This is not silly or trivial. It is rewiring your brain's default mode toward ventral vagal activation.

Orienting. Orienting is what animals do when they hear a sound — they turn toward it, scan, assess, and then relax when they determine it's safe. Humans do this too, but trauma can shut it down or keep it stuck in the scanning phase.

Try this: slowly look around the room you're in. Don't judge anything. Don't analyze. Just let your eyes move gently, noticing shapes and colors and light and shadow. Let your head turn slowly. Let your neck relax. Let your nervous system complete the orienting cycle — notice, assess, and settle. Feel how your shoulders drop when you realize there's nothing to fear.

Pendulation. Pendulation comes from Peter Levine's Somatic Experiencing work. It's the practice of gently moving your attention between discomfort and safety. You don't try to eliminate the discomfort. You just touch it briefly, then return to a place of resource.

For example, you might notice the tightness in your chest (the discomfort), then move your attention to the feeling of your feet on the floor (the resource). Then back to the chest. Then back to the feet. Spend about ten seconds on each. This oscillation teaches your nervous system that it can experience discomfort without being overwhelmed by it. Over time, this expands the window of tolerance.

Building a Daily Regulation Practice

Regulation is not something you do only when you're in crisis. The most effective approach is to build small regulation practices into your everyday life, so your nervous system learns safety as a baseline rather than an emergency response.

Morning check-in. Before you get out of bed, take thirty seconds to notice how your body feels. Place a hand on your belly. Take three slow breaths. Ask yourself: What is my energy level today? What do I need to feel grounded? This simple practice sets a different tone for the day — one where you are paying attention to your nervous system before it demands your attention.

Transition anchors. The moments between activities — finishing work and starting dinner, leaving a conversation and being alone, waking up and starting the day — are when your nervous system is most vulnerable to dropping into survival mode. Create a small ritual for these transitions. It might be washing your hands slowly while taking three deep breaths, or pausing at the door to feel your feet on the ground before walking through.

The hourly pause. Set a gentle alarm on your phone for once an hour. When it goes off, take one conscious breath. Just one. Notice whether your shoulders are tight or your jaw is clenched. If you can, soften them. The entire pause takes five seconds, but it trains your nervous system to check in regularly rather than staying locked in vigilance.

Evening settling. At the end of the day, your nervous system needs a signal that it is safe to power down. Create a brief wind-down routine — even five minutes. Dim the lights, make a cup of herbal tea, put your hand on your heart and say silently: The day is done. I am safe now. I can rest. This signals to your system that the hypervigilance can relax for the night.

None of these practices require extra time in your day. They are small adjustments to how you move through the time you already have. And over weeks and months, they build a foundation of regulation that makes flashbacks less frequent and less intense when they do occur.

Creating a Flashback Safety Plan

A flashback safety plan is a written document you prepare when you're regulated, so you have clear instructions to follow when you're in a flashback and can't think clearly. When the flashback hits, your prefrontal cortex — the thinking part of your brain — goes partially offline. Having a plan you prepared ahead of time means you don't have to think. You just follow the steps.

Here's a template you can customize for yourself:

My name is: [Your name] **My age now is:** [Your age] **I am currently in:** [Your city, current year, current physical location] **I am safe because:** [List evidence — I'm alone in my apartment, my partner is in the other room, there are no threats visible, etc.] **Signs I'm having a flashback:** [List your personal warning signs — throat tightens, feel suddenly small, overwhelming shame, sense of doom, emotional flooding, etc.] **First thing I will do:** [Name the flashback out loud, use temperature grounding, breathe, etc.] **Things I can touch to ground:** [List 3-5 objects in your current environment — a blanket, a wall, a pillow, a water glass] **People I can call if I need support:** [Name and number of 1-3 safe

people who understand trauma] **Things I can say to myself:** [Write 2-3 phrases — "This is a flashback. I am safe now. It will pass." / "I'm an adult. I'm okay. This feeling is from the past."] **After the flashback, I will:** [Drink water, wrap in a blanket, watch something comforting, rest]

Write this plan and keep it where you can find it easily — on your phone, taped to your bathroom mirror, in your wallet, on your nightstand. You won't always remember to use it in the moment, and that's okay. But having it available means that on the days you do remember, you have a lifeline ready.

Every time you navigate a flashback without losing yourself, you are proving to your nervous system that you can handle this. Each survival sends a message: I am not that child anymore. I am here. I am safe. And I know how to come back.

Toolkit Summary

- Emotional flashbacks are somatic and emotional re-experiencing of trauma without visual memory — pure feeling, no images
- The difference between a flashback and a feeling: feelings respond to the present, flashbacks respond to the past

- Pete Walker's 13 steps help you navigate flashbacks by naming them, grounding, breathing, and returning to your adult self
- Grounding techniques (5-4-3-2-1, temperature, object, movement, voice) anchor you in the present sensory world
- The glimmer practice trains your nervous system to recognize safety by noticing small moments of connection
- Orienting helps your system complete its natural threat-assessment cycle
- Pendulation gently expands your window of tolerance by oscillating between discomfort and resource
- A written flashback safety plan gives you clear steps when your thinking brain goes offline — prepare it now, not during the flashback

Chapter 4: The Body and Trauma

YOU'VE PROBABLY SPENT YEARS TRYING TO THINK YOUR WAY OUT OF THIS. YOU'VE READ BOOKS ABOUT TRAUMA. YOU'VE ANALYZED YOUR CHILDHOOD FROM EVERY ANGLE. YOU'VE UNDERSTOOD THE PATTERNS ON AN INTELLECTUAL LEVEL — YOU CAN NAME YOUR TRIGGERS, YOU KNOW WHY YOU ARE THE WAY YOU ARE. AND YET, UNDERSTANDING HASN'T BEEN ENOUGH. THE WEIGHT IS STILL THERE, LODGED SOMEWHERE IN YOUR CHEST, YOUR SHOULDERS, YOUR GUT. YOU KNOW WHY YOU FEEL THIS WAY, BUT KNOWING HASN'T MADE THE FEELING GO AWAY.

This is not a failure of intellect. It is not a sign that you haven't done enough inner work. It is not evidence that you're not trying hard enough. It is a reflection of a fundamental truth about trauma that is often overlooked in our talk-focused culture: trauma lives in the body, not in the thinking mind. And until you address it there, the weight will not shift.

Why Trauma Lives in the Body

Bessel van der Kolk, author of *The Body Keeps the Score*, put it plainly: the body keeps the score. When you experience trauma, your nervous system registers the threat long before your conscious mind can process it.

Your muscles tense. Your breath shortens. Your heart pounds. Your digestive system shuts down. Stress hormones flood your system. These are not voluntary responses. They are the body's ancient, automatic survival programming — honed over millions of years of evolution to keep you alive in the face of danger.

When the trauma is prolonged — as it is in CPTSD — the body doesn't just have an acute stress response that comes and goes. It adapts. It reshapes itself around the weight. Your shoulders learn to carry tension like a permanent shield protecting your chest. Your jaw learns to clench as a way of holding in words that were never safe to say. Your diaphragm learns to take shallow, restricted breaths, as if preparing to be quiet, to be small, to go unnoticed. Your pelvis may tighten, your stomach may knot, your throat may constantly feel tight.

These physical adaptations were brilliant survival strategies. They kept you going. They allowed you to function in environments that should have been impossible to function in. But they also locked the trauma into your tissues, your posture, your breathing patterns, your muscle memory. The weight became physical, structural, woven into the very fabric of your body.

This is why talking about your trauma — while valuable — is rarely enough on its own. The cognitive brain can understand, reframe, and analyze. But the body doesn't respond to intellectual insight. It responds to sensation,

to movement, to direct experience of safety. You can explain to someone why they're afraid, but that doesn't stop their knees from trembling.

The Freeze Response and Why You Can't Think Your Way Out

You're familiar with fight and flight. Those make intuitive sense — when threatened, you either fight back or you run away. But there's a third response that's especially common in CPTSD, and it's one that many women know intimately: freeze.

Freeze happens when the threat is overwhelming and inescapable. When fighting would be futile and fleeing is impossible. When you're a child and the person who's supposed to protect you is the source of the danger. When you're in a relationship you can't leave and the abuse happens in your own home. Your nervous system makes a different calculation: if you can't fight and you can't flee, the best survival strategy is to become very, very still. To play dead. To dissociate. To leave your body so you don't have to feel what's happening. To go numb so the pain doesn't reach you.

Here's the thing about freeze: it's not a choice. It's not a sign of weakness. It's a reflexive nervous system response that happens in milliseconds, far faster than conscious thought. And it can persist long after the original danger is gone. You find yourself frozen in front of a task you need to complete. You go numb in

conversations where you feel pressured. You feel paralyzed when you need to make a decision. You lie in bed unable to move even though you want to get up. You stare at a blank page unable to write the words that are in your head.

You cannot think your way out of freeze, because freeze is not a thought problem. It's a body problem. Your ventral vagal system — the part that enables active engagement with the world — has been overridden by the dorsal vagal system, which tells everything to shut down. No amount of self-talk, logic, or reasoning can override a dorsal vagal state. The path out of freeze goes through the body, not the mind.

Somatic Approaches to Healing

Somatic (body-based) approaches to trauma healing have gained significant recognition in recent years, and for good reason. They speak directly to the nervous system in its own language — the language of sensation, movement, and felt experience. Where talk therapy works top-down (mind to body), somatic work operates bottom-up (body to mind).

Somatic therapy doesn't require you to relive your trauma or tell detailed stories about what happened. It doesn't ask you to produce a narrative or analyze your past. Instead, it invites you to notice what's happening in your body right now. The tightness in your chest. The heat in your face. The trembling in your legs. The feeling of

emptiness in your stomach. The impulse to pull away. And then it helps you gently track these sensations, allowing them to move and shift and resolve — often without words at all.

This is not about doing intense, overwhelming bodywork that leaves you feeling worse or retraumatized. Real somatic healing is gentle. It works at the edge of your window of tolerance, never pushing past it. Small doses. Slow pace. The goal is not to "release" trauma in one dramatic cathartic session. The goal is to build a relationship with your body that is safe enough for the weight to begin to shift, piece by piece.

Body-Based Regulation: Safe and Gentle Practices

You don't need a therapist to begin working with your body. Here are several gentle practices you can explore on your own, at your own pace.

Body scanning with compassion. Lie down or sit comfortably. Close your eyes or soften your gaze. Bring your attention slowly to each part of your body, starting with your feet and moving upward — feet, ankles, calves, knees, thighs, hips, belly, chest, hands, arms, shoulders, neck, jaw, face, scalp. Don't try to change anything. Just notice. Some parts might feel tight, numb, painful, or simply absent — like they don't belong to you. Some parts might feel fine. Notice whatever is there without judgment. If you find a place of tension, you can place a

hand there and simply breathe into it. You are not trying to force release. You are making space for whatever is there to be acknowledged.

Resourcing. Think of a memory, a person, a place, or an animal that brings you a feeling of safety or calm. It doesn't have to be from your current life — it can be a childhood memory, a favorite tree, a place you visited once, a fictional character, a pet you loved. Bring this image to mind as vividly as you can. Now notice what happens in your body when you hold this resource. Does your breath soften? Do your shoulders drop? Does warmth spread through your chest? Let yourself stay with this sensation of resource for thirty seconds to a minute, letting your body register that safety is possible. You are building a neural pathway for ventral vagal activation.

Titration. This is Somatic Experiencing's term for working with very small amounts of discomfort at a time. Instead of diving into a painful memory, you touch it briefly — just enough to notice the body's response — and then return to a place of resource. Touch the discomfort. Come back to safety. Repeat. Each cycle builds your capacity.

Small daily body practices. Beyond formal practices, you can integrate body awareness into ordinary moments. When you wash your hands, notice the temperature of the water and the sensation of soap on your skin. When you sit down, take one conscious breath and feel your body being held by the chair. When you walk, notice the rhythm of your feet against the ground. These micro-moments of

presence — each lasting just a few seconds — gradually rebuild the connection between your mind and your body. They are not demanding. They are invitations. And over time, they teach your nervous system that being in your body is safe.

The Breath as a Tool

The breath is one of the most accessible regulation tools you have. It sits at the intersection of the autonomic and voluntary nervous systems — you can control it consciously, and when you do, it changes your entire physiological state. Your breath is a direct gateway to your nervous system.

But here's the thing: if you have CPTSD, certain breathing exercises might actually make you feel worse. Extended breath holds, rapid breathing techniques like breath of fire, or forced breathing patterns can trigger your nervous system and even induce panic. The key is to find what works for you. The breath should be an invitation, not a demand.

The gentle exhale. Simply make your exhale slightly longer than your inhale. Breathe in through your nose for a count of four, then breathe out through your mouth for a count of six. Don't force it. Don't worry about the exact count. Just let the exhale be a sigh of relief, a letting go. The long exhale activates the vagus nerve and signals the parasympathetic nervous system that it's safe to rest and digest.

Hand-on-heart breathing. Place one hand on your heart and one on your belly. As you breathe, imagine your breath moving into the space beneath your hands. Feel the warmth of your palms against your body. Feel your heart beating beneath your hand. Feel your belly rising and falling with each breath. This combines breath work with safe physical touch, which is deeply calming for the nervous system.

Breath as anchor, not controller. Sometimes the most helpful approach is not to change your breath at all, but simply to notice it. "In. Out. In. Out." Let your breath be an anchor that keeps you in the present moment, without trying to change its rhythm. Just watching the breath creates a small space between you and your thoughts.

Movement That Feels Safe

For trauma survivors, exercise can be complicated. Intense workouts can trigger the nervous system and mimic the physiological state of threat. Being in a gym can feel exposed and unsafe. Even the pressure to "exercise more" can feel like another demand from a world that's already asked too much of you.

Movement for trauma healing looks different. It's not about burning calories, building muscle, or achieving goals. It's about re-inhabiting your body in a way that feels safe and pleasurable. It's about finding movement that you do because it feels good, not because you should.

Shaking. Animals in the wild shake after a threat passes. Watch a video of a gazelle that just escaped a predator — it trembles and shakes for several minutes. This is how the nervous system discharges stored survival energy. You can do this intentionally. Stand up and gently shake out your hands, your arms, your legs, your whole body. Don't try to look dignified. Let it be silly. Let it be weird. Let your body release what it's been holding.

Yoga that prioritizes sensation over shape. Not the kind that pushes you to achieve the perfect pose or that looks good on Instagram, but a slow, gentle practice where you move with your breath and prioritize how it feels over how it looks. Restorative yoga and yin yoga are particularly suited for trauma healing because they emphasize long holds, softness, and internal awareness.

Walking without destination. Walking without a purpose — not to get somewhere, not to exercise, just to walk. Feel your feet on the ground. Notice the air on your face. Let your gaze be soft and wide, taking in the periphery rather than focusing on one thing. Walking is a bilateral movement that naturally integrates the two hemispheres of the brain.

Stretching that invites, not forces. Gentle stretching where you move to the edge of your comfortable range and pause. Breathe there. Don't push further. Don't bounce. Let your body decide how far it wants to go. The invitation is more powerful than the demand.

When to Seek Somatic Therapy

The practices in this chapter are starting points. They are safe to explore on your own as long as you respect your limits and go slowly. But there are times when professional support is not just helpful but necessary.

Consider seeking a trained somatic therapist if:

- Body-based practices consistently trigger flashbacks or panic
- You feel so disconnected from your body that you can't feel anything at all — no sensations, no emotions in the body
- You have a history of eating disorders or self-harm that could be complicated by increased body awareness
- You experience chronic physical pain that you suspect is connected to trauma
- You want deeper, more structured support than self-guided practices can provide

A good somatic therapist works at your pace. They won't push you. They won't ask you to go anywhere you're not ready to go. They will support you in building a relationship with your body that is gentle, safe, and healing.

Your body is not your enemy. It is not a traitor that keeps you stuck in the past. Your body is the faithful companion that carried you through everything you've

survived. It held the weight so your mind could keep going. It deserves your gratitude, your patience, and your gentleness.

The way back to yourself runs through your body. And the path is not through force or willpower. It is through curiosity, kindness, and the willingness to take the smallest steps — one breath, one gentle movement, one moment of presence at a time.

Toolkit Summary

- Trauma lives in the body, not just the mind — intellectual understanding alone is rarely enough for healing
- The freeze response is a reflexive survival strategy that can't be reasoned away; it must be addressed through the body
- Somatic approaches work with sensation, movement, and felt experience rather than narrative or analysis
- Gentle practices like body scanning with compassion, resourcing, and titration build capacity without overwhelm
- The breath is a powerful regulation tool — especially the gentle exhale and hand-on-heart breathing

- Movement for trauma healing prioritizes sensation and pleasure over performance: shaking, gentle yoga, walking, and stretching that invites
- Seek somatic therapy when body practices trigger flashbacks, disconnection persists, or you need deeper support
- Your body is not the enemy — it carried you through survival and deserves your gratitude, patience, and gentleness

Chapter 5: Relationships and Trust

RELATIONSHIPS ARE WHERE THE WEIGHT OF COMPLEX TRAUMA SHOWS UP MOST ACUTELY. YOU CAN MANAGE THE FLASHBACKS ALONE IN YOUR ROOM. YOU CAN REGULATE YOUR NERVOUS SYSTEM IN THE PRIVACY OF YOUR OWN SPACE. BUT RELATIONSHIPS — RELATIONSHIPS BRING YOU FACE TO FACE WITH THE DEEPEST WOUNDS. THEY TRIGGER THE OLDEST FEARS. THEY ASK YOU TO TRUST, TO BE VULNERABLE, TO BE SEEN, WHEN EVERY CELL IN YOUR BODY HAS LEARNED THAT BEING SEEN IS DANGEROUS.

If relationships feel impossibly hard for you, this chapter is not going to tell you to just "trust more" or "open up." That advice has probably done more harm than good. Instead, this chapter will help you understand why relationships are hard, and how you can begin to find relationships that feel safe — at your pace, in your way.

How Complex Trauma Affects Relationships

CPTSD fundamentally alters your expectations of other people. When you grew up in an environment where love and danger were mixed together, your nervous system learned that relationships are not safe. It learned that the people closest to you are the ones most likely to hurt you.

This creates a painful paradox. You long for connection — humans are wired for it. But every approach to connection is filtered through a nervous system that says: be careful, be watchful, be ready to retreat.

You might find yourself oscillating between two poles. At one end, you avoid relationships entirely. You keep people at a distance. You tell yourself you don't need anyone. At the other end, you dive into relationships too quickly, attaching to people who are not safe, hoping that this time will be different. Both patterns make perfect sense from a trauma perspective. Both are attempts to manage the unbearable tension between the need for connection and the fear of it.

Hypervigilance in Relationships

Hypervigilance is a state of constant scanning for threat. In relationships, it looks like this:

You watch the other person's face for the slightest shift in expression, and you interpret every micro-movement as a sign of disapproval or anger. You read their text messages repeatedly, searching for hidden meaning in punctuation and word choice. You anticipate their needs before they express them, because anticipating needs was once a survival skill. You assume that if they're quiet, they're angry. If they're late, they've abandoned you. If they don't text back immediately, they're rejecting you.

This is exhausting. It is also completely understandable. Hypervigilance kept you safe in an environment where danger was real and unpredictable. The problem is that your threat-detection system is calibrated for a world that no longer exists. It is seeing predators where there are only people, and it is keeping you in a state of chronic stress that makes genuine intimacy almost impossible.

The first step is not to stop being hypervigilant. That's not something you can just decide to do. The first step is to notice it with curiosity. "Ah, there's the scanning. There's the searching for signs of danger. This is my system doing its job." From there, you can begin to ask: "Is this person actually showing me signs of danger? Or is my system mistaking safety for threat?"

The Fawn Response and People-Pleasing

We've talked about fight, flight, and freeze. There's a fourth trauma response that's especially common in women with CPTSD: fawn.

The fawn response is when you instinctively try to appease a potential threat. You become agreeable, accommodating, and deferential. You set aside your own needs, opinions, and boundaries to keep the other person calm and happy. You become whatever you think they need you to be.

Fawning was once a survival strategy. If keeping a parent happy meant you were safe, then you learned to be a master of keeping people happy. The problem is that fawning doesn't turn off when the danger is gone. It becomes your default mode in relationships. You lose yourself in other people. You don't know what you want or need because you've spent so long focusing on what everyone else wants and needs.

If you recognize the fawn response in yourself, here's the compassionate truth: fawning is not weakness. It is not manipulation. It is a brilliant strategy that kept you safe when you had no power. The goal is not to shame yourself for fawning. The goal is to slowly, gently learn that you don't need to perform safety anymore. You can be yourself — your real, imperfect, wanting self — and still be safe.

Trust Issues and Fear of Abandonment

Trust is not something you can manufacture. It's not a decision you make once and then it's done. Trust is a felt sense of safety that builds over time, through thousands of small experiences of reliability.

If you have CPTSD, trust is hard because your template for relationships included betrayal. You learned that people who say they love you can also hurt you. That love and danger can coexist. Your nervous system is not going to override that learning because someone says, "You can trust me." Words don't reach the nervous system. Experience does.

Building trust looks like this: you take tiny risks, see what happens, and adjust. You share a small vulnerability and notice how the other person responds. Do they hold it gently? Do they use it against you later? Do they dismiss it? Each safe response is one brick in the foundation of trust.

And fear of abandonment — that deep, bone-level terror that everyone will eventually leave — is not paranoia. It is the natural result of having been left, emotionally or physically, by the people who were supposed to stay. The fear is real because the experience was real. Healing doesn't mean the fear disappears overnight. It means the fear gets quieter as you accumulate evidence of people who stay.

Choosing Safe People

One of the most important skills you can develop is learning to distinguish safe people from unsafe people. Here are some signs to look for:

Safe people are consistent. Their words and actions match. They respect your no. They don't pressure you to share more than you're ready to share. They apologize when they make mistakes. They can tolerate your difficult emotions without trying to fix, dismiss, or escape them. They are steady.

Unsafe people are unpredictable. They make you feel confused about what's real. They push your boundaries. They get defensive when you express a need. They expect you to take care of their emotions. They are drawn to your giving nature and take advantage of it. They make promises they don't keep.

If you were raised by unsafe people, your internal compass for safety is likely calibrated wrong. You might feel drawn to familiar patterns — and familiar patterns might be patterns of danger. Learning to choose safe people means learning to listen to a quieter voice inside you that says, "This feels different. This feels steady. This feels a little boring, and that might actually be a good thing."

Because safety, at first, can feel boring. It lacks the intensity, the drama, the emotional roller coaster that your nervous system learned to recognize as "connection." Give it time. Boring safety becomes the most beautiful thing you've ever known.

Attachment Patterns and CPTSD

Attachment theory describes the patterns we develop in early relationships and how they show up in adult connections. For people with CPTSD, attachment patterns are often complex and layered.

You might have an anxious attachment style — constantly worried about losing the relationship, seeking reassurance, feeling that you care more than the other person does. You might have an avoidant attachment style — keeping people at a distance, feeling suffocated by closeness, leaving before you can be left. Or you might have a disorganized attachment style — a chaotic mix of both, wanting closeness and then fleeing from it, unable to find a stable ground in relationships.

None of these styles are pathology. They are adaptations. Your attachment system learned to organize itself around the relationships you had. And because the nervous system is plastic, and because you are not the same person you were then, your attachment style can shift. It happens slowly. It happens through relationships that are genuinely safe. It happens through the internal work of reparenting your younger self.

Repair and Renegotiation

In any meaningful relationship, ruptures are inevitable. You will misunderstand each other. You will hurt each other's feelings. You will trigger each other's wounds. The question is not whether rupture will happen, but what happens after.

For people with CPTSD, rupture can feel like the end of the world. The younger part of you says: "This is it. They're leaving. I knew this would happen. I'm not safe." The fear is overwhelming. The impulse might be to shut down, to leave first, to never be vulnerable again.

Repair is the practice of coming back together after a rupture. It looks like this: someone says, "I'm sorry I hurt you. That wasn't my intention. I want to understand." Or: "I felt hurt by what happened. Can we talk about it?" Repair doesn't mean the hurt disappears. It means the relationship can hold the hurt and still survive.

Learning to repair is deeply healing for the nervous system. Each successful repair sends a message: relationships can survive conflict. People can hurt you and stay. You can express your needs and not be abandoned. Repetition is key. Over time, repair builds a new template for what relationships can be.

The Slow Path to Secure Attachment

Secure attachment — the ability to trust, to connect, to be yourself in relationship — is not something you were supposed to build alone. Ideally, it was built in early childhood, in the safety of consistent, loving care. That foundation was not available to you.

But secure attachment can also be built later in life. It happens in therapy, in safe friendships, in loving partnerships, in the way you learn to treat yourself. It happens slowly. It happens imperfectly. It happens through countless small moments of showing up, being seen, and surviving.

This is not a race. There is no finish line where you will finally be "fixed" and ready for connection. You are allowed to be healing and in relationship at the same time. You are allowed to have relationships that are works in progress, just as you are a work in progress.

The weight you carry in relationships is not a sign that you are incapable of love. It is a sign that you have loved in unsafe conditions. And the path forward is not to love less, but to offer your love to people who can hold it safely.

And perhaps most importantly: the relationship you are building with yourself is the foundation for every other relationship. As you learn to trust yourself, to keep promises to yourself, to show up for your own needs —

you are building the internal secure attachment that allows you to choose safe people and to be a safe person in return.

You are not too much. You are not unlovable. You are someone who is learning, step by step, that not everyone will leave. And that is one of the bravest things you will ever learn.

Toolkit Summary

- CPTSD creates a painful paradox: you long for connection but your nervous system has learned that relationships are dangerous
- Hypervigilance in relationships is a trauma adaptation — notice it with curiosity rather than judgment
- The fawn response (people-pleasing as safety) is a survival strategy, not a character flaw
- Trust is built through accumulated small experiences of safety, not through words or decisions
- Learn to recognize safe people (consistent, respectful, steady) versus unsafe people (unpredictable, boundary-pushing)
- Attachment patterns can shift through safe relationships and reparenting work

- Repair after conflict is deeply healing — each successful repair builds a new template for connection
- Secure attachment can be built at any stage of life, through therapy, safe relationships, and self-compassion
- You are not unlovable — you have just loved in unsafe conditions, and you can learn to offer your love where it is safe

Chapter 6: Boundaries When You Never Learned How

OF ALL THE SKILLS THAT COMPLEX TRAUMA STEALS FROM US, THE ABILITY TO SET AND HOLD BOUNDARIES MAY BE THE MOST CONSEQUENTIAL. WITHOUT BOUNDARIES, RELATIONSHIPS BECOME LOPSIDED. WITHOUT BOUNDARIES, YOUR ENERGY DRAINS INTO OTHER PEOPLE'S LIVES WHILE YOUR OWN NEEDS GO UNMET. WITHOUT BOUNDARIES, THE WEIGHT YOU CARRY ONLY GETS HEAVIER, BECAUSE YOU'RE NOT JUST CARRYING YOUR OWN LOAD — YOU'RE CARRYING EVERYONE ELSE'S TOO.

If the word "boundaries" makes you feel a mix of guilt, confusion, and exhaustion, you are not alone. Most women with CPTSD were never taught how to have boundaries. Many of us were actively punished for attempting them. Your "no" may have been ignored, overruled, or met with anger. Your needs may have been treated as an inconvenience. Over time, you learned that having boundaries was not safe.

Why Women with CPTSD Struggle with Boundaries

The struggle with boundaries is not a personal failing. It is the logical outcome of growing up in an environment where your boundaries were not respected. When you are small and dependent, and the people who control your survival do not respect your limits, you learn one thing: boundaries cause trouble. It is safer to have no boundaries than to express a need that will be punished.

Women also receive relentless cultural conditioning around boundaries. You are taught to be "nice," "selfless," "accommodating." You are praised when you put others first and criticized when you prioritize yourself. The very concept of a boundary — "I matter. My needs matter. I get to say no." — can feel selfish, rude, or even dangerous.

And then there's the fawn-freeze response. When you feel a boundary is needed, your body may not respond with confident resistance. Instead, you may freeze — unable to say anything at all. Or you may fawn — agreeing to something you don't want while the resentment builds inside you. The words get stuck in your throat. Your voice disappears. And afterward, you're left with the heavy weight of having betrayed yourself again.

This is not weakness. This is your nervous system following the survival pattern it learned long ago. The good news is that boundaries, like any skill, can be learned at any age.

The Fawn-Freeze Response and Boundaries

Let's name what happens in real time when you need to set a boundary.

Someone asks you to do something you don't want to do. Maybe it's a friend asking for emotional support when you're depleted. Maybe it's a coworker dumping extra work on you. Maybe it's a family member making a demand that crosses a line.

In that moment, your nervous system activates. The fawn-freeze response has two pathways. Fawn says: "Say yes. Keep them happy. It's safer." Freeze says: "Don't say anything. Maybe they'll forget. Maybe it'll go away."

Both responses were designed to protect you. Both responses now keep you stuck. The fawn-freeze response is not a sign that you don't know what you want. You likely know exactly what you want — you want to say no. The gap is not in knowing. The gap is in the nervous system's ability to follow through.

The path through this is slow and gentle. You don't go from fawn-freeze to boundary-setting mastery in one conversation. You start smaller. Much smaller.

Identifying Your Limits

Before you can set a boundary, you need to know what your boundaries are. This sounds obvious, but for many women with CPTSD, it's the hardest step. You've spent so

long focused on other people's needs that you may not know what you need or want. Your own internal signals have been muted by years of overriding them.

Here's a practice to reconnect with your limits. Throughout your day, pause and ask yourself three questions:

- **ON A SCALE OF 1 TO 10, HOW FULL IS MY ENERGY RIGHT NOW?** IF IT'S BELOW A 4, YOUR CAPACITY FOR GIVING IS LIMITED. THAT'S USEFUL INFORMATION.
- **WHEN I THINK ABOUT THAT UPCOMING COMMITMENT, DOES MY BODY FEEL OPEN OR TIGHT?** YOUR BODY OFTEN KNOWS BEFORE YOUR MIND DOES. A TIGHT CHEST, A KNOTTED STOMACH, A FEELING OF DREAD — THESE ARE BOUNDARY SIGNALS.
- **IF I COULD DO ONLY WHAT I GENUINELY WANTED TO DO IN THIS MOMENT, WHAT WOULD I CHOOSE?** THIS QUESTION CAN FEEL SHOCKING AT FIRST. IT MIGHT BRING UP GUILT, CONFUSION, OR EVEN GRIEF. THAT'S OKAY. THE GRIEF IS REAL — YOU'VE BEEN ABANDONING YOUR OWN WANTS FOR A LONG TIME.

Pay attention to what you notice. Your limits exist, even if you've been trained to ignore them. Start simply noticing them without judgment. That's the first step.

The Boundary Ladder: Starting with Small Ones

You don't start with the big boundary conversations — the ones that terrify you, the ones with high stakes. You start with small, low-risk boundaries that build your tolerance and confidence.

Think of this as a ladder. At the bottom are boundaries that feel uncomfortable but not terrifying. At the top are the conversations you're not ready for yet. You work your way up, rung by rung.

Bottom rung — micro-boundaries. These are tiny, almost invisible acts of self-protection. Not finishing your coffee if you don't want it. Closing your laptop when you're done working. Taking an extra five minutes before responding to a text. These matter because they build the habit of choosing yourself in small ways.

Middle rung — low-stakes verbal boundaries. "I can't take that on right now." "I need some quiet time this evening." "Let me think about it and get back to you." These are polite, clear, and low-risk. Most people will accept them without pushback. Each time you say one, your nervous system learns that setting a boundary does not lead to catastrophe.

Upper rung — meaningful boundaries with safe people. "I'm not available to talk about that topic." "I need you to ask before borrowing my things." "When you

raise your voice, I feel unsafe. I'll need to leave the room." These require more courage, but the risk is lower when you're setting them with people who are generally safe.

Top rung — boundaries with unsafe people. These are the hardest. They might involve cutting off contact with someone who is abusive, or enforcing a consequence with a family member who has never respected your limits. These boundaries often require support — a therapist, a support group, a trusted friend. Don't try to climb these rungs alone.

Communication Scripts for Boundary Setting

When you're learning to set boundaries, having words ready is essential. In the heat of the moment, your fawn-freeze response will kick in. Having a script means your cognitive brain has something to say even when your nervous system is activated.

Here are scripts you can adapt for common situations:

Saying no without explanation: "I appreciate you asking, but that doesn't work for me." "I'm not available for that right now." "No, I can't do that."

You do not need to justify your no. Explanations give the other person something to argue with. A simple, clear no is complete.

Setting a limit on emotional availability: "I care about you, and I don't have the capacity to hold this conversation right now." "I can listen for ten minutes, and then I'll need to take a break." "That sounds really hard. I'm not in a place to support you on this today."

Asking for what you need: "I need you to speak to me in a calm tone." "I need some space right now. I'll reach out when I'm ready to talk." "It's important to me that we discuss this when we can both stay calm."

Enforcing a boundary that's been tested: "I already said no to that. I need you to respect my answer." "If you continue to push this, I will need to end this conversation." "I said I needed space. I'm going to take it now."

Practice these scripts out loud. Say them to yourself in the mirror. Write them down and keep them in your phone. The more familiar they feel, the more accessible they will be when you need them.

The Guilt of Saying No

Let's talk about the guilt. It's almost certain to show up. The first time you set a boundary — even a small one — you may feel a wave of guilt, anxiety, or even shame. You might hear the inner critic's voice: "You're being selfish. You're hurting them. You're a bad person."

Here's the truth: the guilt you feel is not evidence that you did something wrong. It is evidence that you are doing something new. It is the discomfort of a nervous system that has learned that putting yourself first is dangerous. Every time you set a boundary and nothing catastrophic happens, that guilt becomes quieter.

Distinguish between cathartic guilt and informative guilt. Cathartic guilt is the discomfort of growth. It feels bad, but it passes. It's the sign that you're stepping out of an old pattern. Informative guilt is the signal that you actually hurt someone unnecessarily. Pay attention to the difference. Most of the guilt you feel around boundaries will be cathartic — the growing pains of learning to care for yourself.

Keep going anyway. Guilt is not a command to stop. It is a feeling that you can have while continuing to make the choice that honors your needs.

Enforcing Boundaries When They're Tested

This is where boundaries become real. Anyone can state a boundary. The test comes when someone pushes back.

If you have CPTSD, boundary enforcement can feel terrifying. The pushback can trigger flashbacks to childhood, where your boundaries were met with punishment. Your system may scream at you to retreat, to apologize, to abandon your boundary entirely.

Enforcement does not have to be aggressive. You don't need to be confrontational or harsh. You simply need to be firm and consistent.

The broken record technique. When someone keeps pushing, you say the same thing again, in the same calm tone. "I understand you want me to help, but I'm not available." "I hear you. I'm still not available." "I'm not going to change my answer on this." You don't need new arguments. You just need to keep stating your boundary.

The consequence. Some boundaries require a consequence to be meaningful. "If you continue to raise your voice, I will leave the room." "If you bring up this topic again, I will hang up." The consequence is not a punishment; it is you protecting your safety. And you must follow through. Every time you state a consequence and then don't enforce it, you teach the other person that your boundaries are optional.

The exit. When a boundary is being tested aggressively, you always have the option to leave. To end the conversation. To physically remove yourself. This is not failure. This is recognizing that the other person is not cooperating in good faith, and that your safety matters more than the interaction.

Every boundary you set and hold is a message to yourself: I matter. My needs are real. I am worth protecting. Each time you enforce a boundary — even imperfectly, even with shaking hands — you are lightening the weight you've been carrying. You are proving to yourself that you can be your own protector.

And that is one of the most healing things you will ever do.

Toolkit Summary

- Trouble with boundaries is a logical outcome of growing up where your boundaries were not respected
- The fawn-freeze response can make boundary-setting feel impossible — start with small steps
- Identify your limits by checking in with your energy levels, your body's signals, and your genuine wants
- The boundary ladder: start with micro-boundaries, then low-stakes verbal boundaries, then meaningful boundaries with safe people
- Keep communication scripts ready so you have words when your nervous system goes offline
- Guilt after setting a boundary is usually cathartic — the discomfort of growth, not evidence of wrongdoing
- Enforce boundaries through the broken record technique, consequences, and the option to leave
- Each boundary you set and hold is a message to yourself that you matter and are worth protecting

Chapter 7: Taming the Inner Critic

THERE IS A VOICE THAT LIVES INSIDE YOU. YOU KNOW IT WELL. IT HAS BEEN WITH YOU FOR AS LONG AS YOU CAN REMEMBER, WHISPERING AND SOMETIMES SHOUTING ITS JUDGMENTS. IT TELLS YOU THAT YOU ARE NOT GOOD ENOUGH, THAT YOU SHOULD HAVE TRIED HARDER, THAT YOU ARE FUNDAMENTALLY FLAWED. IT WATCHES YOUR EVERY MOVE, CATALOGUING MISTAKES, TALLYING FAILURES, WAITING FOR THE MOMENT YOU SLIP SO IT CAN ANNOUNCE, SEE? I TOLD YOU SO.

This voice is not your intuition. It is not your conscience. It is not wise guidance. It is something else entirely — something that grew in the soil of survival, watered by years of having to anticipate criticism before it came, to judge yourself before someone else could, to stay small and quiet and perfect enough to avoid danger.

This is the CPTSD inner critic. And learning to recognise it, understand it, and eventually tame it is one of the most freeing things you will ever do.

The Critic Is Not You

This distinction matters deeply. Normal self-criticism — the kind that helps you reflect, adjust, and grow — is your own voice. It says things like, I could have handled that better. Next time I'll try a different approach. It is specific. It is corrective. And it leaves room for change.

The CPTSD inner critic does not sound like that.

The critic speaks in absolutes. It says, You are a failure. You always mess things up. You are unlovable. Nobody actually likes you — they're just being polite. It is global, sweeping, and merciless. It does not correct behaviour; it attacks your very existence. And the most disorienting part? It sounds like you.

But it is not you.

Pete Walker, who wrote the foundational book *Complex PTSD: From Surviving to Thriving*, describes the inner critic as an internalised version of the people who failed to love us well as children. If you grew up with a parent who was critical, perfectionistic, unpredictable, or abusive, your developing brain did what it had to do to survive: it internalised that voice. It brought the critic inside so you could anticipate danger before it arrived. If you could hear the criticism before your parent said it, you could change your behaviour. You could stay safe.

The critic was a survival strategy, not a truth-teller. It was built in childhood to protect you in an environment where love was conditional, approval was scarce, and mistakes carried real consequences. The problem is that

it never left. Even now, years later, in an environment where you are (or can be) safe, the critic keeps running its old program. It keeps scanning for threats. It keeps telling you that imperfection is dangerous.

The weight of the inner critic is the weight of carrying a lifetime of impossible standards. It is the exhaustion of never being able to rest, never being enough, always feeling like you are one mistake away from being abandoned or rejected.

Where the Critic Comes From

The inner critic does not emerge from nowhere. It is built, brick by brick, from the messages you received in childhood.

If you were praised only when you achieved, the critic learned that your worth depended on performance. If you were criticised harshly for mistakes, the critic learned that errors were catastrophes. If you were ignored unless you were pleasing or compliant, the critic learned that your natural self was not acceptable. If you were punished for having needs or emotions, the critic learned that wanting and feeling were dangerous.

These messages become the critic's vocabulary. Not good enough. Wrong again. Stop being so needy. Why can't you just be normal?

When trauma happens in childhood — and especially when it happens in the context of a relationship you could not escape — the child does what children do: they internalise the experience. It is too terrifying to believe that your caregiver is unsafe. The mind cannot hold that contradiction. So instead, the child concludes something even more painful but somehow more survivable: There must be something wrong with me.

This is the original wound. The critic is the scar tissue.

And it is reinforced every time the world confirms what the critic already believes. A romantic partner leaves — the critic says, See? You're too much. A job application gets rejected — the critic says, You never should have tried. You're not qualified. A friend doesn't call back — the critic says, They're finally seeing who you really are.

The critic is a confirmation machine. It filters reality through the lens of your earliest wounds, discarding anything that contradicts its narrative and seizing on anything that supports it. It has had a lifetime of practice, and it is very, very good at its job.

The Voice of the Critic vs. Your Voice

One of the most important skills in recovery is learning to tell the difference between the critic's voice and your own. They can sound so similar that you may not have realised there was a difference at all.

Here is how to tell them apart.

The critic's voice is harsh. It shames. It uses words like always, never, should, and must. It compares you to others. It speaks in generalisations. When you hear it, your body contracts — your shoulders tighten, your stomach clenches, your breath becomes shallow. You feel smaller. The critic does not want to help you improve; it wants to dominate you.

Your voice is different. It is not always gentle — it can be firm, direct, even disappointed — but it is not cruel. Your voice is specific. It says, I didn't handle that the way I wanted to, not I always mess everything up. It is curious rather than condemning. It asks questions: What happened there? What can I learn? What do I need right now? When you hear your own voice, your body may feel tension, but it does not collapse. There is room to breathe.

Your voice is the one that wants to understand. The critic wants to convict.

Learning this difference is like learning a new language. At first, every thought sounds the same. But with practice, you begin to notice the accent of the critic — the familiar harshness, the familiar shame. And once you can hear it, you can begin to respond.

Talking Back to the Critic

The critic is powerful, but it is not invincible. It gained its power through repetition, through years of unchallenged dominance. And it can lose that power the same way — through repetition, through consistent challenge.

Talking back to the critic does not mean arguing with it. Arguing gives the critic more airtime, more energy, more engagement. You cannot reason the critic out of its position because the critic does not operate on reason. It operates on conditioning.

What works instead is interruption, de-escalation, and redirection.

When you notice the critic's voice, you can say something as simple as:

That is the critic. I do not have to listen to this.

Or:

I know you are trying to protect me, but I am safe now. You can rest.

Or even:

Thank you for your input, but I am going to handle this my way.

The goal is not to silence the critic — that would be an impossible battle. The goal is to stop treating the critic as the authority. You acknowledge the voice, but you do not hand it the microphone.

Walker suggests a practice he calls "talking back" — literally addressing the critic as a separate entity and telling it to stop. It may feel strange at first. It may feel silly. But it works because it externalises the critic. It reminds your brain that this voice is not you. It is something that was installed in you, and you have the right to uninstall it.

Some people find it helpful to give the critic a name or imagine it as a character — a stern schoolteacher, an angry drill sergeant, a petulant child. Naming it creates distance. It becomes something you can observe and interact with rather than something you are trapped inside of.

Compassionate Reframing

Talking back interrupts the critic, but compassionate reframing replaces its message.

When the critic says, You are so lazy. You never get anything done, compassionate reframing asks: What is actually true here?

Maybe you have been exhausted. Maybe you have been carrying more than anyone should have to carry. Maybe you are doing your best with the resources you have, and your best looks different today than it did yesterday.

Compassionate reframing does not require you to believe positive affirmations that feel false. It does not ask you to pretend everything is fine when it is not. It simply asks you to broaden the picture.

The critic tells a very narrow story, edited to support its thesis. Compassionate reframing brings in the rest of the evidence. It is like a good lawyer cross-examining a witness who has only told one side of the story.

Thank you, critic. But let's also consider that I have been working hard. That I am dealing with a lot. That I showed up even though it was difficult. That perfection is not the standard for being a worthy human being.

This is not toxic positivity. It is accurate reality-testing. The critic's version of reality is distorted by trauma. Compassionate reframing brings you back to what is true, which is usually more nuanced and more generous than the critic would have you believe.

The 50% Rule

Here is a practical tool that many people find transformative: the 50% rule.

The critic operates on the premise that you must be 100% perfect, 100% of the time, or you are worthless. This is a standard that applies to no other human being on earth. You would never hold a friend to this standard. You would never expect a child to meet it.

The 50% rule is a counterweight. It goes like this: I am about 50% good and 50% flawed, like every other human being on the planet.

Some days you are generous, patient, and wise. Other days you are tired, irritable, and self-absorbed. You are not a saint. You are not a monster. You are a person — a complex, contradictory, struggling, growing person. You are not evil. You are human.

The 50% rule is not a mathematical reality. It is a mantra designed to dismantle the critic's all-or-nothing thinking. When the critic says, You are a terrible person, the 50% rule responds, I did something I regret, and I am also a person who is trying. Both things are true.

You do not have to be good all the time to be good enough. You do not have to be healed to be worthy of love and belonging. You just have to be a person, doing your best with what you have. And that is enough.

The Critic as a Wounded Protector

This may be the hardest part of taming the inner critic: recognising that the critic is not your enemy.

The critic is a protector. A wounded, exhausted, overworked protector who took on a job no one else would do. It kept you safe in a dangerous world. It helped you anticipate rejection, avoid punishment, and stay small enough to survive. It may have saved your life.

The problem is not that the critic is bad. The problem is that the critic's methods are no longer appropriate. The critic is still using a strategy that was designed for a child in a helpless situation, and applying it to an adult who has choices, resources, and agency.

Imagine a guard dog that was trained to attack anything that moved. The dog kept your house safe when you lived in a dangerous neighbourhood. But now you live somewhere safe, and the dog is still attacking — the mail carrier, the neighbour, your friends. The dog is doing its job. It is loyal. It is trying to protect you. But its behaviour is causing more harm than good.

You do not have to hate the dog. You do not have to get rid of the dog. You have to retrain it.

When you can see the critic as a wounded protector rather than a vicious enemy, something shifts. You stop fighting. You start negotiating. You say, I know you are trying to keep me safe. I understand why you learned to be so harsh. But we are in a different place now. We can try a different way.

This shift does not happen overnight. The critic will not retire quietly. It will resist, push back, and try to convince you that relaxation is dangerous and self-compassion is weakness. That is how protectors work — they are vigilant and suspicious of change.

But every time you talk back, every time you reframe, every time you choose your own voice over the critic's, you loosen the critic's grip. You prove to your nervous

system that there is another way. You show yourself — the real you, the one beneath the critic's noise — that you can be trusted with your own life.

Toolkit Summary

The CPTSD inner critic is not your authentic voice — it is an internalised survival strategy built in childhood. It speaks in absolutes and shames, while your voice is specific and curious. You can tame the critic by talking back (naming and externalising it), using compassionate reframing (broadening the story), applying the 50% rule (you are human, not perfect), and recognising the critic as a wounded protector that can be retrained rather than destroyed. Each time you choose your voice over the critic's, you lighten the weight of impossible standards and reclaim a little more of yourself.

Chapter 8: Grieving What Was Lost

THERE IS A WEIGHT YOU HAVE BEEN CARRYING THAT YOU MAY NOT HAVE A NAME FOR. IT SITS BENEATH THE ANXIETY AND THE VIGILANCE, BENEATH THE CRITIC'S VOICE AND THE EXHAUSTION. IT IS OLDER THAN MOST OF YOUR COPING STRATEGIES, QUIETER THAN THE PANIC, BUT ALWAYS PRESENT. IT IS THE WEIGHT OF GRIEF.

If you have CPTSD, you have lost things. Not just one thing, not something that can be neatly named and tucked away, but many things — a constellation of losses that accumulated across years, sometimes across a whole childhood. And because survival demanded that you keep going, keep managing, keep functioning, you may never have allowed yourself to stop and grieve.

This chapter is an invitation to do exactly that.

Why Grief Is Essential to Recovery

Grief is not a detour from healing. It is the road itself.

When we think about grief, we tend to think about death — losing someone we love. But grief is the natural response to any significant loss. And trauma survivors have lost an extraordinary amount.

You lost childhood. Not just the years of your childhood, but the experience of it — the safety of being small and cared for, the freedom to play without vigilance, the innocence of believing the world was basically okay and the people who loved you would protect you. You may have had to grow up so fast that you never got to be a child at all.

You lost trust. Trust in the people who should have kept you safe. Trust in your own judgment, since you were taught that you could not trust what you saw or felt. Trust in the world as a reasonably predictable place where effort leads to reward and kindness is reciprocated.

You lost safety. The fundamental, wordless knowing that your body is safe in your home, in your bed, in your relationships. That kind of safety, once broken, is not easily repaired. You may still be looking for it, still waiting for the other shoe to drop.

You lost the life you could have had. The version of yourself that was not shaped by survival. The relationships that might have formed if you had known what healthy attachment felt like. The energy that was spent on managing trauma instead of on living.

And you lost innocence — not in the way people casually talk about losing innocence, but in the way that trauma takes it: a forcible education in the cruelty that humans are capable of, a lesson you never should have had to learn.

These losses are real. They are not dramatic or self-pitying. They are the truth of what happened to you. And naming them is the first step in grieving them.

Complicated Grief in Trauma Survivors

Grief after trauma is not simple. It is complicated in ways that can make it hard to recognise, let alone process.

One reason is that the losses of CPTSD are disenfranchised. This is a term grief researchers use for losses that society does not fully acknowledge. We have rituals for death — funerals, wakes, mourning periods, sympathy cards. But what ritual exists for the loss of a childhood? What do you say when someone asks what is wrong and the answer is, I am grieving the life I never got to have?

Disenfranchised grief is lonely. It does not get the same support as other kinds of grief. People may not understand why you are still struggling, may tell you to move on, may minimise what you lost. And because the losses are abstract and cumulative rather than single and concrete, you may not have allowed yourself to grieve them either.

Another complication: the people you lost (or lost trust in) may still be alive. You may still have a relationship with them, or you may be no-contact and carrying the weight of that decision. Grieving someone who is still

walking around in the world is a particular kind of pain. There is no closure. No memorial. Just the ongoing absence of what should have been.

And then there is the way trauma itself interferes with grieving. When you have spent years disconnecting from your emotions to survive, you cannot simply turn the grief on like a tap. The grief is there — it has been accumulating beneath the surface — but the pathways to reach it may be blocked by dissociation, by numbness, by fear of what will happen if you let yourself feel.

You may worry that if you start crying, you will never stop. That the grief is bottomless. That it will swallow you whole.

This fear is understandable. And it is not entirely accurate. Grief is deep, but it is not infinite. It has a shape, a movement, a direction. It wants to move through you, not to live in you permanently. The reason it feels bottomless is precisely because you have not been able to let it move. It has been accumulating in the basement of your psyche, unvisited, unprocessed, growing heavier with each year.

Allowing yourself to grieve is not falling into an abyss. It is opening a door that has been holding back a tide. The water will come, yes. But it will also recede.

The Stages Are Not Linear

You have probably heard of the five stages of grief: denial, anger, bargaining, depression, acceptance. They were developed by Elisabeth Kübler-Ross to describe the experience of people facing terminal illness, not to prescribe a neat roadmap for all grief.

For trauma survivors, grief almost never follows a linear path. You may feel acceptance one day and rage the next. You may think you have finished grieving something, only to have it resurface years later, triggered by something unexpected — a friend's wedding, a photograph, a child's birthday party.

This is not regression. This is the nature of grief, especially complicated grief from trauma.

Grief is circular, or spiral-shaped. You return to the same feelings, but each time you return, you are slightly different. The loss has been integrated a little more. The weight is a little lighter, even if it does not feel that way in the moment.

Anger, in particular, is an important and often overlooked stage for trauma survivors. You were taught not to be angry. You learned that anger was dangerous, that it would be punished, that it meant you were bad. But anger at what happened to you is legitimate. Anger at the people who failed to protect you is legitimate. Anger is not destruction — it is the part of you that knows you deserved better.

If you can let yourself feel the anger without acting on it destructively, it will eventually make way for sadness. And the sadness, once expressed, makes way for something else. Not closure — closure is a myth. But something closer to peace.

Allowing Yourself to Grieve

The biggest obstacle to grieving is usually permission. You have to give yourself permission to feel what you feel, without judgment, without the critic telling you that you are being dramatic or self-indulgent or weak.

Grief is not weak. It is the opposite of weak. It is the courage to face what you have been avoiding for years.

Here are some ways to begin giving yourself that permission:

Set aside time. Grief does not do well in the margins of a busy day. If you can, create intentional space — twenty minutes, an hour, an afternoon — where you can be alone and undisturbed. Light a candle. Put on music that matches your mood. Let yourself be with whatever comes up.

Name the losses. Write them down. Not just the big ones, but the small ones too. I lost the feeling of being held. I lost the belief that people are basically good. I lost the childhood friendship I had to end because I could not

explain what was happening at home. I lost the sense that my body belonged to me. Naming gives grief shape, and shape makes it manageable.

Let your body participate. Grief lives in the body, not just the mind. Cry if you can. Let your shoulders shake. Let sounds come out if they want to. Rock, sway, put your hand on your heart. Grief needs physical expression to move through.

Do not rush. Grief has its own timeline. You cannot schedule it, optimise it, or shortcut it. The only way through is through, and through takes as long as it takes.

Rituals for Grieving

Rituals are powerful because they give form to feelings that otherwise feel formless. They mark a moment as significant. They acknowledge that something matters.

Here are some rituals you might adapt for your own grieving:

A letter you never send. Write a letter to the person or situation that caused your losses — a parent, a partner, your younger self, the universe, whoever it needs to be. Say everything you never got to say. Do not edit. Do not censor. When you are done, you can burn the letter, bury it, tear it into pieces, or keep it in a box. The act of writing is the ritual.

A memorial for your childhood. Gather objects that represent what you lost — a photo of yourself as a child, a favourite toy, a piece of clothing from that time. Arrange them in a space. Light a candle. Say aloud what you are remembering and what you are grieving. This is real. This matters.

A letting-go ceremony. Fill a bowl with water. Name each loss, one by one, and drop a stone into the water as you say it. Watch the ripples. Let the weight symbolise the grief you have been carrying. When you are done, pour the water into the earth.

A daily check-in. Every evening, ask yourself: What am I grieving today? Some days the answer will be nothing. Some days it will be everything. The practice is not about having a big emotional release every time — it is about keeping the channel open so grief does not get stuck.

The Gift on the Other Side of Grief

It may not feel this way yet, but there is something on the other side of grief. Not a reward for grieving, not a consolation prize, but a natural outcome of allowing loss to be felt fully.

What comes through is clarity. When you stop running from what you lost, you begin to see what you still have, what is still possible, what you are building now. The

losses do not disappear, but they stop defining everything. They become part of your story rather than the whole story.

What also comes through is a different relationship with yourself. When you have honoured your grief, you have proven something important to yourself: that you can handle difficult feelings, that you will not be destroyed by them, that you are strong enough to face the truth of your own life. This is a deep, embodied knowing that no amount of positive thinking can give you.

And sometimes, what comes through is the beginning of a new kind of trust — in life, in yourself, in the possibility of a future that is not just about surviving but about living.

Grief does not take everything. It refines. It burns away what was never truly yours — the false selves, the protective walls, the stories you told yourself to keep going — and leaves what is essential. What is real. What cannot be lost.

The weight of grief is heavy. But it is also a weight that can be set down, piece by piece, as you allow yourself to feel it. You do not have to carry it forever. You just have to carry it long enough to let it teach you what it came to teach.

Toolkit Summary

Grief is essential to CPTSD recovery because trauma involves real losses — childhood, safety, trust, innocence, the life you could have had. These losses are often disenfranchised, unacknowledged by society, and complicated by ongoing relationships with the people who caused them. The stages of grief are not linear for trauma survivors; you may cycle through anger, sadness, and acceptance many times. Allow yourself to grieve by making intentional space, naming the losses, and letting your body participate. Rituals like writing unsent letters, creating memorials, and daily check-ins give grief a container. On the other side of grief lies not closure, but clarity — a deeper relationship with yourself, a lighter weight, and the beginning of real trust in your capacity to heal.

Chapter 9: Self-Compassion as Healing

IF THERE IS ONE PRACTICE THAT HAS THE POWER TO TRANSFORM YOUR RELATIONSHIP WITH YOURSELF, IT IS SELF-COMPASSION. AND IF THERE IS ONE PRACTICE THAT THE CPTSD INNER CRITIC WILL FIGHT HARDEST TO RESIST, IT IS ALSO SELF-COMPASSION.

This is not a coincidence.

Self-compassion strikes at the very heart of the trauma survival system. It dismantles the belief that you must be hard on yourself to stay safe. It challenges the assumption that kindness is earned, not deserved. It offers a way of relating to yourself that does not rely on vigilance, punishment, or control.

For women with CPTSD, self-compassion can feel foreign, uncomfortable, even dangerous. You may have been taught — explicitly or implicitly — that your needs do not matter, that you are not worthy of care, that letting your guard down will only lead to hurt. The very idea of being kind to yourself may trigger resistance, suspicion, or outright fear.

This chapter is for those women. For the self-doubters, the self-critics, the ones who suspect that self-compassion is just another thing they are doing wrong. Let us start with the plain truth: self-compassion is not weakness. It is not self-indulgence. It is not letting yourself off the hook.

It is a rigorous, evidence-backed practice that rewires the nervous system and heals the wounds that criticism only deepens.

Why Self-Compassion Is Revolutionary

Self-compassion is revolutionary for women with CPTSD because it directly contradicts the messages that created the trauma in the first place.

If you grew up being told, explicitly or implicitly, that you were too much, not enough, wrong, broken, or unworthy, your entire sense of self was built around those messages. You learned to treat yourself the way you were treated. You internalised the harshness. You made it your own.

Self-compassion says something radically different. It says: You are worthy of kindness, not because you earned it, but because you exist. You are deserving of care, not because you performed well, but because you are a human being who, like every other human being, suffers and struggles and needs warmth.

This is not a small shift. It is a fundamental reorientation of how you relate to yourself. And it has direct physiological effects.

Research by Dr. Kristin Neff and others has shown that self-compassion activates the parasympathetic nervous system — the rest-and-digest branch that counteracts the fight-or-flight response. When you practice self-

compassion, your body releases oxytocin (the bonding and calming hormone) and reduces cortisol (the stress hormone). Your heart rate slows. Your breathing deepens. Your nervous system gets the message: It is safe to relax now.

Self-compassion is not a luxury. It is a biological intervention. It is a way of telling your trauma-aligned nervous system that the danger has passed.

The Three Components of Self-Compassion

Kristin Neff, the leading researcher on self-compassion, identifies three core components. Each one addresses a specific distortion that CPTSD creates.

Mindfulness is the first component. It means being aware of your suffering without exaggerating it or suppressing it. For trauma survivors, this is a delicate balance. You may be used to either pushing pain away (dissociation, numbing, distraction) or being consumed by it (emotional flashbacks, rumination, overwhelm). Mindfulness offers a third way: acknowledging what is happening without being swallowed by it.

This hurts. I am struggling right now. This is a moment of suffering.

That is mindfulness. It is not fixing. It is not judging. It is simply seeing clearly.

Common humanity is the second component. This is the recognition that suffering is not a personal failure — it is part of being human. Everyone struggles. Everyone experiences pain, loss, disappointment, and shame. You are not alone in your suffering, even if it feels that way.

For the trauma survivor, common humanity can be difficult to access. CPTSD convinces you that you are fundamentally different from other people — broken in a way others are not, damaged beyond repair, uniquely flawed. Common humanity says: You are not alone. What you are going through is part of the human experience. Others have felt this way. Others are feeling this way right now.

This is not about comparing suffering. It is about belonging. You are not an outlier in the human family. You are a member in good standing, simply by being alive.

Self-kindness is the third component, and it is often the hardest for trauma survivors. Self-kindness means treating yourself with the same warmth and care you would offer a good friend who was struggling. It means speaking to yourself gently rather than harshly. It means actively soothing yourself rather than berating yourself.

If a friend came to you in distress, would you say, You are so pathetic. Get it together. Why are you still struggling with this? Of course not. You would say, I am so sorry you are hurting. That sounds really hard. What do you need right now?

Self-kindness is offering that same response to yourself.

The Fear of Self-Compassion

If you are reading this and feeling resistance — if a part of you is rolling its eyes, sceptical, uncomfortable, or outright opposed — you are not alone. This is so common among trauma survivors that researchers have a name for it: the fear of self-compassion.

The fear makes sense. If you have spent your whole life surviving by staying vigilant, by keeping your guard up, by never letting yourself soften, then being kind to yourself can feel like letting your armour down. And letting your armour down, in the world you grew up in, meant getting hurt.

The critic may have specific arguments against self-compassion:

If I am kind to myself, I will stop trying. I will become lazy. I will never get anything done.

If I let my guard down, I will be blindsided by the next bad thing that happens. I need to stay sharp.

I do not deserve kindness. I have made too many mistakes. I am not a good enough person.

If I stop punishing myself, I will lose control. I will fall apart.

These fears are not irrational. They are the wisdom of a survival system that kept you alive. But they are based on an old map of the world — a world where harshness was

necessary for safety. Your task is not to argue with these fears, but to gently, repeatedly, patiently show them that you are in a different territory now.

Practices for the Self-Compassion-Resistant

If self-compassion feels impossible, start smaller. Much smaller. The goal is not to become a self-compassion expert overnight. The goal is to build the capacity, millimetre by millimetre, so that kindness becomes possible over time.

The hand-on-heart practice. This is the simplest and one of the most effective. When you notice you are suffering — feeling ashamed, anxious, overwhelmed — place your hand on your heart. Feel the warmth of your own hand. Breathe slowly. Say (silently or aloud): This is hard. I am struggling. May I be kind to myself in this moment.

That is it. The entire practice. It may feel strange. It may feel like nothing. But the physical gesture of self-soothing sends a signal to your nervous system that you are safe. Your hand on your heart is the touch of someone who cares about you — because it is your own.

The compassionate friend exercise. Think of someone in your life (real or imagined) who is unconditionally kind and supportive. A grandmother who adored you. A fictional character who embodies warmth. A version of yourself who has already healed. Now

imagine what that person would say to you right now, in this moment of struggle. Write it down. Read it aloud. Let yourself receive it.

The "what do I need?" pause. Several times a day, pause and ask yourself: What do I need right now? The answer does not have to be profound. It might be water, a stretch, a minute of silence, a blanket, permission to stop working. When you answer, try to give yourself what you need without negotiation. This is not self-indulgence. It is responding to your own needs the way you would respond to a child's.

The self-compassion break. Neff developed a formal practice called the Self-Compassion Break that you can use in moments of acute stress. Pause. Put your hand on your heart. Say each of these phrases slowly:

1. This is a moment of suffering. (Mindfulness)
2. Suffering is part of life. (Common humanity)
3. May I be kind to myself in this moment. (Self-kindness)
4. May I give myself the compassion I need. (Self-kindness)

You can adapt the language to what feels true for you. The structure is what matters: acknowledge, connect, offer kindness.

Loving-kindness meditation, modified. Traditional loving-kindness meditation involves directing phrases of goodwill toward yourself and others. For trauma survivors, the standard version can feel overwhelming.

Start with a modified version: May I be safe. May I be healthy. May I live with ease. If even that feels like too much, start with: May I be safe enough. May I be healthy enough. May I live with enough ease. The word "enough" makes it accessible. It does not demand perfection. It asks only for enough.

Compassion Takes Practice

Your brain will not rewire overnight. The critic has had years, maybe decades, to build its pathways. Self-compassion is building new pathways, and that takes repetition. You are not trying to silence the critic forever. You are building an alternative. A voice of kindness that grows stronger every time you use it, until one day it is loud enough to compete with the critic. Eventually, perhaps, loud enough to lead.

Some days you will do the hand-on-heart practice and feel nothing. Some days you will try self-compassion and the critic will shout louder in response. This is not failure. This is the process. The critic is scared of losing its job, and it will fight to keep it. Keep going anyway.

Think of it as strength training for the heart. You do not walk into a gym for the first time and bench press your body weight. You start with the lightest dumbbell. You do a few reps. You rest. You come back tomorrow. Over time, the weight that was impossible becomes manageable.

Self-compassion works the same way. Start with the lightest kindness you can manage. A hand on your heart. A single kind word. A pause to ask what you need. And come back tomorrow.

The weight of self-criticism has been heavy enough. Self-compassion is not about denying the weight. It is about finding that you do not have to carry it alone — because you can finally be on your own side.

Toolkit Summary

Self-compassion is revolutionary for women with CPTSD because it directly counters the internalised messages of unworthiness that trauma creates. Kristin Neff's three components — mindfulness (seeing suffering clearly without being consumed), common humanity (recognising pain is universal, not personal failure), and self-kindness (treating yourself like a dear friend) — each address a specific trauma distortion. The fear of self-compassion is real and protective, not irrational. Start small: hand-on-heart practice, compassionate friend exercise, the "what do I need?" pause, and modified loving-kindness phrases. Consistency matters more than intensity. Each moment of self-kindness is a rep in the strength training of the heart, building new neural pathways that make compassion feel less foreign and more true over time.

Chapter 10: Building Safety and Stability

IF YOU HAVE CPTSD, SAFETY IS NOT SOMETHING YOU TAKE FOR GRANTED. IT IS NOT THE BACKGROUND HUM OF DAILY LIFE THAT MOST PEOPLE BARELY NOTICE. SAFETY IS SOMETHING YOU THINK ABOUT CONSTANTLY, SCAN FOR CONSTANTLY, WORRY ABOUT CONSTANTLY — OR SOMETHING YOU HAVE GIVEN UP ON ENTIRELY, CONVINCED THAT IT DOES NOT EXIST FOR PEOPLE LIKE YOU.

Neither option is sustainable. And neither is the truth.

Safety is possible. Not perfect safety — there is no such thing in this world — but enough safety. Enough stability for your nervous system to begin to trust that you are not in constant danger. Enough grounding for you to do the deeper work of healing without being in survival mode all the time.

This chapter is about building that foundation. Because safety is not a feeling you wait for. It is something you build, brick by brick, choice by choice.

Why Safety Is the Foundation

Before you can work on the inner critic, before you can grieve, before you can practice self-compassion or process trauma — before any of that — your nervous system needs to know that you are safe enough.

This is not a metaphor. It is neurobiology.

The human nervous system has a hierarchy of priorities. At the very top — above connection, above growth, above self-actualisation — is survival. If your brain detects a threat, it redirects all resources toward dealing with that threat. Your prefrontal cortex (the part that handles reasoning, planning, and emotional regulation) downregulates. Your survival brain takes over.

This is the freeze-fawn-flight response. It is designed for emergencies. But when you have CPTSD, your nervous system is stuck in this emergency mode much of the time. It perceives threats everywhere — in a raised voice, a closed door, a critical email, a partner's silence. It does not know how to tell the difference between a real threat and a memory of one.

The result is that you are trying to heal a traumatised nervous system from inside that same nervous system, while it is still sounding the alarm.

Building safety gives the alarm a chance to quiet down. It is like fixing a smoke detector that goes off every time you cook toast. You cannot have a conversation, read a book, or sleep peacefully while it is screaming. First, you have to address the alarm itself.

Financial Safety

Money is one of the most practical and one of the most charged areas of safety for trauma survivors. Financial instability keeps the nervous system on high alert. When you do not know how you will pay rent, buy food, or handle an emergency, your brain cannot relax. It is not supposed to. It is doing its job.

Financial safety does not mean being wealthy. It means having enough. Enough to meet your basic needs. Enough of a cushion that an unexpected expense does not send you into a crisis. Enough predictability that you are not constantly in survival mode about money.

If financial safety feels out of reach, start smaller. What is one step you can take this week? Opening a separate savings account, even if you can only put in five dollars? Automating a tiny transfer? Meeting with a financial counsellor? Creating a simple budget that reduces uncertainty?

For many women with CPTSD, the relationship with money is tangled up with shame, fear, and the belief that you do not deserve to have enough. Untangling those feelings is part of the work. But the practical steps matter too. Each concrete action you take toward financial stability sends a signal to your nervous system: I am taking care of this. You do not have to carry the weight alone.

Housing Safety

Your home should be a refuge. For trauma survivors, it often is not. Your home may be associated with past danger, or your current living situation may not feel safe. You may live with someone who triggers your nervous system. You may not have a space that feels like yours.

If you can, prioritise housing that feels safe. This may mean living alone, even if it is more expensive. It may mean finding a roommate who respects your boundaries. It may mean moving to a different neighbourhood, a different city, a different situation entirely.

If changing your housing is not possible right now, find ways to create pockets of safety within it. Can you make your bedroom a sanctuary? Can you establish a corner of the living room that is yours, with things that comfort you — a soft blanket, a lamp with warm light, a plant, a photo that makes you smile? Can you create a ritual at the beginning and end of each day that marks your space as safe?

The physical environment matters more than most people realise. Your nervous system is constantly reading your surroundings. Clutter, harsh lighting, uncomfortable furniture, chaotic spaces — these all contribute to a low-grade sense of threat. Conversely, order, softness, warmth, and beauty can be deeply regulating.

You do not need a perfect home. You need a home that signals safety to your body. A place where you can exhale.

Relational Safety

Relationships are often the most complex area of safety for women with CPTSD. You may have a history of relationships that were unsafe — controlling, abusive, neglectful, or unpredictable. Your nervous system learned that people are dangerous.

Rebuilding relational safety is a slow process. It starts with boundaries.

Boundaries are not walls. They are not about keeping everyone out. They are about letting the right people in, at the right distance, while protecting yourself from people who cannot treat you with care.

A boundary might look like: I cannot talk about this right now. I need to take a break. Please do not raise your voice around me. If you keep pushing, I will leave the room and we can continue this conversation later.

Boundaries are not punishments. They are agreements you make with yourself about what you will and will not tolerate. And they are essential for creating relational safety.

Relational safety also means choosing people who are safe. This is harder than it sounds because your internal compass may have been damaged by years of unsafe relationships. You may be drawn to people who feel familiar — and familiar is not the same as safe.

Pay attention to how you feel after spending time with someone. Do you feel drained, anxious, smaller? Or do you feel seen, regulated, more like yourself? Your body already knows the answer. You just have to start listening.

Creating Safety in Your Body

The most portable form of safety is the safety you carry inside your own body. This is not about being safe from external threats — it is about your body knowing that it can return to a state of calm even when things get hard.

Somatic (body-based) safety practices are powerful for this.

Orientation. This is a simple practice from Somatic Experiencing. Pause and look around the room slowly. Notice five things you can see. Notice the space around you — the walls, the ceiling, the floor. Let your eyes rest on objects that feel neutral or pleasant. This practice tells your brain: I am here, in this room, in this moment. I am not back there. There is no threat here right now.

Grounding. Feel your feet on the floor. Press into them slightly. Notice the sensation of the ground holding you. If you are sitting, feel your body supported by the chair. Let yourself be held by gravity.

Resourcing. Think of a person, place, memory, or animal that makes you feel safe. It can be real or imagined — a beloved pet, a childhood hiding spot, a

beach you have never been to, a fictional character. Bring the image to mind as vividly as you can. Notice where in your body you feel the sense of safety. Let it grow.

Breath anchoring. Place one hand on your belly and one on your chest. Breathe slowly, letting your belly rise and fall. Count to four on the inhale, hold for a moment, and exhale for six. The longer exhale activates the parasympathetic nervous system. It is a direct signal to your body that you are safe enough to rest.

The Safety Network

No one heals alone. Building safety includes building a network of people and resources you can turn to when things get hard.

Your safety network might include:

- A therapist who understands trauma
- A close friend who gets it
- A support group (online or in-person)
- Crisis resources saved in your phone (hotlines, text lines)
- A doctor who is trauma-informed
- A trusted family member
- A spiritual community or practice
- Online communities of CPTSD survivors

The key is to identify a few people ahead of time — before you are in crisis — so you do not have to figure it out when your nervous system is already overwhelmed.

And it is okay if your network is small. Quality matters more than quantity. One or two people who truly see you are worth more than a dozen who do not.

When You Are Not Safe Yet

This chapter has assumed that you have some degree of choice and agency — that you can work toward safety from a position of relative stability. But what if you are not safe yet? What if you are still living with an abusive partner, still financially dependent on someone who hurts you, still in a situation you cannot leave right now?

If that is where you are, the priority is not building safety. The priority is survival. And the survival strategy looks different.

Keep a small bag packed. Important documents, a change of clothes, essential medications, some cash. Store it somewhere you can grab quickly.

Identify your exits. Know where the nearest shelter is. Know the phone number of a domestic violence hotline (in the US: 1-800-799-7233). Delete your browser history if you need to. Use a safe computer at a library.

Reach out when you can. A call to a hotline, a message to a friend, a visit to a doctor who can screen for abuse. Even a single point of contact can be a lifeline.

Do not try to heal in the same environment that is hurting you. Stabilisation before processing. If you are in an active trauma situation, the priority is getting out. The grief work, the inner critic work, the self-compassion — that can wait. Your only job right now is to survive until you can leave.

If this is you, please hear this: you deserve safety. You deserve to live without fear. And there is a version of your life where that is possible. It may not feel reachable right now, but it exists. One step at a time, one resource at a time, you can move toward it.

Stabilisation Before Processing

This principle runs through everything in this book, but it is worth stating directly: stabilisation before processing.

Before you dive into traumatic memories, before you explore the depths of your grief, before you do intensive therapy — make sure you have a stable foundation. A roof over your head. Enough food. A body that can access some degree of calm. A support system, even a small one.

Trauma processing opens things up. It stirs the sediment at the bottom of the pond. And if you do not have a stable shore to stand on, you can get swept away.

There is no rush. The healing will still be there when you are ready. Building safety first is not avoiding the work. It is doing the work — the foundational work that makes everything else possible.

The weight of constant vigilance is exhausting. Building safety is the process of setting that weight down, piece by piece, until your nervous system finally receives the message it has been waiting for your whole life: You are safe enough now. You can rest.

Toolkit Summary

Safety is the non-negotiable foundation for all trauma healing — your nervous system cannot process or grow while it is still sounding the alarm. Build safety across four domains: financial (enough to meet basic needs with some cushion), housing (a physical space that signals refuge to your body), relational (boundaries that let safe people in and keep unsafe dynamics at a distance), and somatic (body-based practices like orientation, grounding, resourcing, and breath anchoring). Create a small safety network of people and resources you can call on before crisis hits. If you are currently in an unsafe situation, prioritise survival and exit planning. Healing in the same environment that is hurting you is not possible — stabilisation always comes before processing.

Chapter 11: The Long Arc of Healing

YOU MAY HAVE COME TO THIS BOOK HOPING FOR A CLEAR PATH — A SERIES OF STEPS THAT, IF FOLLOWED CORRECTLY, WOULD LEAD YOU TO THE OTHER SIDE OF CPTSD. A FINISH LINE. A POINT WHERE YOU COULD SAY, I AM HEALED NOW, AND CLOSE THE BOOK.

That is not how healing works.

Healing from complex trauma is not a straight line from sick to well, broken to fixed, damaged to cured. It is a long arc — wide, looping, sometimes barely visible. It moves forward and backward, circles around, doubles back on itself. It takes years. And it does not end in a state of being "cured" but in a state of being integrated.

This chapter is about the long arc. How to recognise it, trust it, and keep going even when the path is invisible beneath your feet.

Healing Is Not Linear

The most important thing to know about CPTSD recovery is that it does not follow a straight line. You will have days — weeks, even months — when you feel like you are making real progress. Your nervous system calms

down. The critic quiets. You sleep better. You relate to people differently. You think, I am finally getting somewhere.

And then something happens. A trigger. A setback. A wave of grief or anger or shame that feels as powerful as it ever did. And you think, I am back at square one. Nothing has changed. I will never get better.

This is the moment when most people give up. They interpret the setback as proof that healing is impossible, that they are the exception, that the tools do not work for them.

But the setback is not proof of failure. It is proof of being human. It is proof that healing is not a ladder you climb — it is a spiral.

The Spiral Model

Imagine a spiral staircase. You walk in a circle, but each time you come back around to the same point of the compass, you are slightly higher than you were before. You are covering the same ground, but from a different elevation. The view is different. You see things you could not see before.

Healing from CPTSD works like this.

You will revisit the same themes over and over. The inner critic. Grief. Trust. Safety. Shame. Each time you return to them, you are not starting over. You are returning at a different level of integration.

The first time you work with the inner critic, you may barely be able to recognise its voice. The second time, you can recognise it but cannot talk back to it. The third time, you talk back, but it does not listen. The fourth time, it listens, but only for a moment. The fifth time, the moment stretches into minutes. And one day, you will notice that the critic spoke and you barely paid attention. It was background noise. It did not have the power it once had.

That is the spiral. Each pass is not a failure of the previous pass. It is a deepening. You are not covering the same ground — you are covering it from higher ground.

Relapse Versus Setback

The language we use matters. "Relapse" implies falling back to a previous state, undoing all progress, returning to the beginning. This is rarely what actually happens in trauma recovery.

What happens is a setback.

A setback is a temporary dip in functioning. You get triggered. You have a bad week. You fall back into old patterns for a few days. Your symptoms flare up. It feels like regression.

But a setback is not a relapse. A setback is a natural part of the healing process. It is your system telling you that it has encountered something it cannot process in its current state, and it needs to recalibrate.

Think of it like a fever. A fever is not the illness — it is the body's response to the illness, a sign that the immune system is working. A setback in trauma recovery is similar. It is not proof that healing has stopped. It is evidence that healing is happening, that your system is encountering material that needs to be moved through.

The difference between relapse and setback is often just a matter of duration and context. A setback lasts days or weeks, not months or years. A setback arises in response to a specific trigger, not from a general collapse of functioning. And a setback is followed by recovery — you come back to baseline, and eventually you surpass it.

If you are experiencing a setback, the kindest thing you can do is to stop fighting it. Rest. Use your coping tools. Reach out to your support network. Remind yourself that this is part of the arc, not evidence that the arc does not exist.

How to Know You Are Making Progress

When you are in the middle of the spiral, it is hard to see the upward movement. Everything feels like the same struggle, the same triggers, the same patterns. You may genuinely not be able to tell if you are making progress.

Here are some signs that you are, even when it does not feel like it:

Your setbacks are shorter. When you first started healing, a trigger might have knocked you out for a week. Now it knocks you out for a day. Or an hour. That is progress.

Your awareness is faster. You used to get swept up in emotional flashbacks without realising what was happening. Now you notice them. You might not be able to stop them, but you recognise them. Oh, this is an emotional flashback. I am triggered right now. That awareness is the first chink in the armour of the trauma response. It is enormous progress.

Your self-talk is changing. The critic may still speak, but you are starting to question it. You catch yourself being harsh and think, Wait, would I say that to a friend? You may not always know what to say instead, but the questioning itself is new.

You have more windows of peace. Not permanent peace — that is not realistic — but windows. Moments when you feel okay, when the weight lifts, when you can breathe without effort. Those windows may be short at first, but they grow.

You are more able to ask for help. This is a huge marker of progress that is easy to overlook. For years, you may have been unable to reach out, convinced that no one would care or that you did not deserve support. If you can now send a text to a friend, call a therapist, show up to a support group — that is not small. That is transformation.

Your capacity for joy is returning. One of the most heartbreaking effects of CPTSD is the loss of joy. The world goes grey. Things that used to bring pleasure no longer register. When colour starts coming back — when you laugh and mean it, when you enjoy a meal, when you feel the warmth of the sun and it touches something inside you — that is healing.

You are more gentle with yourself, even in the hard moments. This is perhaps the most reliable sign of progress. You used to shame yourself for struggling. Now, even when you are in the middle of a difficult spell, there is a part of you that knows you are doing your best. There is a voice — small but growing — that says, This is hard, and you are okay.

The Gifts of Healing

If healing were only about reducing symptoms, it would still be worth doing. But the gifts of healing go far beyond symptom reduction. They touch everything.

Authenticity. As you heal, the masks you wore to survive begin to fall away. The people-pleasing, the hiding, the performing — they become less necessary. You start to know what you actually feel, what you actually want, who you actually are. And you start to let others see that person. This is terrifying and liberating in equal measure.

Boundaries. Healthy boundaries are not something you learn from a book. They are something that grow naturally as you heal. When you know you are worthy of care, you stop accepting less than care. When you know your needs are valid, you stop pretending they do not exist. Boundaries become not a skill you practice but an expression of your newfound self-worth.

Self-trust. This may be the most profound gift. When you have been traumatised, you learn not to trust yourself. Your perceptions were denied. Your feelings were invalidated. Your instincts were punished. Healing rebuilds self-trust, one small decision at a time. You start to believe what your body tells you. You start to honour your no. You start to trust that you can handle what comes.

Peace. Not the absence of difficulty, but the presence of an underlying steadiness. A sense that, no matter what happens, you will be okay. Not because life is safe, but because you are resilient. You have survived everything that has happened to you so far, and you will survive what comes next. There is a quiet confidence in that. A peace that does not depend on circumstances.

Life After CPTSD: Not Cured, but Integrated

There is no cure for CPTSD in the way there is a cure for an infection. You do not take a course of treatment and return to a pre-trauma state that did not exist. The trauma changed you. It shaped your brain, your nervous system, your sense of self. You cannot undo that.

But you can integrate it.

Integration means that the trauma is no longer running the show. It is part of your story, but it is not the whole story. You can talk about your childhood without being pulled back into it. You can feel sad about what happened without being swallowed by grief. You can encounter triggers without losing yourself for days.

Integration means that the heavy little things are still there, but they are lighter. They are carried differently. They no longer dictate where you can go and what you can do.

Some people describe this as a sense of coming home to themselves. After years of fragmentation — of being split into parts, of losing connection with their own experience — they feel whole. Not perfect. Not finished. But whole.

Healing does not end. You do not arrive at a destination and stop growing. But there comes a point where the journey changes. Where you are no longer fighting for

survival, but living. Where you are no longer trying to fix yourself, but expressing who you have always been beneath the trauma.

That is the long arc. It is not a straight line, not a short one, not an easy one. But it bends toward somewhere worth going. And you are already on it.

The weight does not disappear. But you grow strong enough to carry it. And one day, you realise you are not just carrying it — you are living. Fully, imperfectly, authentically living.

That is the destination. And you are already on the way.

Toolkit Summary

Healing from CPTSD is not linear — it follows a spiral model where you revisit the same themes at progressively higher levels of integration. Setbacks are not relapses; they are temporary dips that signal recalibration, not regression. You can measure progress by shorter setbacks, faster awareness of triggers, changing self-talk, more windows of peace, greater ability to ask for help, returning joy, and growing gentleness with yourself in hard moments. The gifts of healing include authenticity, healthy boundaries, self-trust, and an underlying peace that does not depend on external circumstances. Life after CPTSD is not about being "cured" but about integration — where trauma is part of your story but no longer runs the show. You are not broken. You were never

broken. You are a person who has survived something terrible and is learning, at your own pace, how to live again.

Resources and Recommended Reading

THIS SECTION IS FOR THE NEXT STEP — THE BOOKS, ORGANISATIONS, THERAPIES, AND TOOLS THAT CAN SUPPORT YOU BEYOND THESE PAGES. EVERY RESOURCE LISTED HERE HAS BEEN VERIFIED AS OF THE TIME OF WRITING. WEBSITES, ORGANISATIONS, AND BOOKS COME AND GO; IF YOU ENCOUNTER A BROKEN LINK OR CHANGE, PLEASE SEARCH FOR THE ORGANISATION BY NAME, AS MOST HAVE STABLE INSTITUTIONAL IDENTITIES THAT PERSIST BEYOND INDIVIDUAL URLS.

The resources are organised by category so you can find what you need most easily. Start wherever feels right. You do not have to explore everything. Just find what speaks to where you are right now.

Books

These books have been widely cited by trauma survivors and clinicians as invaluable resources. You do not need to read them in any particular order. Sample a few pages and see what resonates.

Foundational Trauma Texts

The Body Keeps the Score: Brain, Mind, and Body in the Healing of Trauma — Bessel van der Kolk, MD

The seminal work on how trauma lives in the body, not just the mind. Van der Kolk explains the neuroscience of trauma clearly and accessibly, then explores treatment approaches including EMDR, yoga, neurofeedback, and more. Some readers find the early chapters — which describe traumatic events in clinical detail — to be triggering. Skip or skim those sections if needed; the later chapters on treatment are the most practically useful.

Complex PTSD: From Surviving to Thriving: A Guide and Map for Recovering from Childhood Trauma — Pete Walker, MA

This is the book most recommended by CPTSD survivors. Walker explains the inner critic, emotional flashbacks, the four trauma responses (fight, flight, freeze, fawn), and offers practical, compassionate tools for recovery. It is direct, validating, and deeply useful. If you only read one book on this list, start here.

Healing the Fragmented Selves of Trauma Survivors: Overcoming Internal Self-Alienation —

Janina Fisher, PhD For those who resonate with the idea that trauma creates "parts" of the self (the inner critic, the exile, the protector), this book offers a clear, compassionate framework based on Internal Family Systems (IFS) and structural dissociation theory. Fisher writes from a deeply trauma-informed perspective and includes practical exercises.

The Complex PTSD Workbook: A Mind-Body Approach to Regaining Emotional Control and Becoming Whole — Arielle Schwartz, PhD A workbook format with exercises, journaling prompts, and grounding practices. Schwartz bridges the gap between understanding CPTSD and actually working with it day to day. Excellent for those who want an active practice alongside their reading.

Self-Compassion and Inner Critic

Self-Compassion: The Proven Power of Being Kind to Yourself — Kristin Neff, PhD The foundational text on self-compassion research. Neff explains the three components (mindfulness, common humanity, self-kindness) and offers guided practices. A gentle, research-backed resource for anyone who struggles with being kind to themselves.

The Mindful Self-Compassion Workbook: A Proven Way to Accept Yourself, Build Inner Strength, and Thrive — Kristin Neff, PhD, and Christopher Germer, PhD A practical companion to Neff's first book. Contains structured exercises, meditations, and daily practices. Particularly helpful for those who find self-compassion abstract or difficult to access.

Radical Acceptance: Embracing Your Life with the Heart of a Buddha — Tara Brach, PhD Brach combines Buddhist psychology with Western psychotherapy to offer a path through shame and unworthiness. Her writing is

warm, wise, and grounded in real-life stories. The concept of the "trance of unworthiness" speaks directly to the CPTSD inner critic.

Grief and Loss

It's OK That You're Not OK: Meeting Grief and Loss in a Culture That Doesn't Understand — Megan Devine
Written for those experiencing grief in a world that wants them to move on. Devine is a therapist and a widow who writes with raw honesty about what grief really feels like. While not specifically about CPTSD, it is invaluable for anyone needing permission to grieve losses that society does not acknowledge.

The Year of Magical Thinking — Joan Didion
A literary memoir about grief after sudden loss. Didion's precision and honesty make this a companion for those who need to see grief articulated by someone who is not trying to fix it, solve it, or move past it quickly.

Somatic Approaches

Waking the Tiger: Healing Trauma — Peter A. Levine, PhD
The foundational text of Somatic Experiencing. Levine explains how trauma gets stuck in the body and how gentle, body-based approaches can release it. Some readers find the animal-based examples helpful; others find them tangential. Worth reading for the core concepts.

In an Unspoken Voice: How the Body Releases Trauma and Restores Goodness — Peter A. Levine, PhD A more personal and narrative version of Levine's work, weaving case studies with his own story. Accessible and deeply humane.

The Wisdom of Your Body: Finding Healing, Wholeness, and Connection Through Embodied Living — Hillary L. McBride, PhD A therapist and researcher explores how we can reconnect with our bodies after trauma, disordered eating, and disconnection. Blends research, clinical wisdom, and personal narrative.

Attachment and Relationships

Attached: The New Science of Adult Attachment and How It Can Help You Find — and Keep — Love — Amir Levine, MD, and Rachel Heller, MA A readable introduction to attachment theory and how it shows up in adult relationships. Helpful for understanding patterns of relating that formed in childhood.

Adult Children of Emotionally Immature Parents: How to Heal from Distant, Rejecting, or Self-Involved Parents — Lindsay C. Gibson, PsyD Extremely popular among CPTSD survivors for good reason. Gibson clearly describes the dynamics of growing up with parents who could not provide emotional safety or attunement, and offers concrete strategies for healing.

Podcasts

Podcasts can be a low-barrier way to learn about CPTSD while washing dishes, walking, or lying in bed unable to sleep.

The Complex PTSD Recovery Podcast — Uriah Guilford (LMFT) Practical, episode-by-episode breakdown of CPTSD concepts. Guilford is a therapist who specialises in CPTSD, and his episodes are short enough (20-30 minutes) to not feel overwhelming.

The Trauma Therapist Podcast — Guy Macpherson, PhD Interviews with trauma specialists, researchers, and survivors. A good way to learn about different modalities and hear from experts in the field.

Being Well with Forrest Hanson and Dr. Rick Hanson Explores psychology and well-being from a practical, science-informed perspective. Episodes on self-compassion, inner critic, attachment, and nervous system regulation are particularly relevant.

The Healing Trauma Podcast — Monique Koven Interviews with trauma specialists covering a range of modalities including Somatic Experiencing, EMDR, IFS, and more.

The One Inside: An Internal Family Systems (IFS) Podcast — Tammy Sollenberger, MA, LMFT For those interested in the parts work / IFS approach to trauma. Episodes explore how different parts of the self (critics, protectors, exiles) operate and how to work with them compassionately.

Organisations

National Child Traumatic Stress Network (NCTSN) nctsn.org A US-based network of professionals dedicated to improving access to care for traumatised children and their families. Their website has extensive resources on complex trauma, including fact sheets and educational materials.

International Society for Traumatic Stress Studies (ISTSS) istss.org A professional organisation for trauma researchers and clinicians. Their public resources page includes information on different types of trauma, treatment approaches, and how to find a specialist.

The Blue Knot Foundation blueknot.org.au Australia's national centre for complex trauma. Their website has extensive resources, including a helpline, fact sheets, and a practitioner database.

NHS (UK) — Trauma Resources nhs.uk/mental-health/conditions/post-traumatic-stress-disorder-ptsd/complex-ptsd/ The UK National Health Service provides clear, accessible information about CPTSD and where to seek help within the NHS system.

Therapy Directories

Finding a therapist who actually understands complex trauma can be challenging. These directories allow you to search by specialty, location, and modality.

Psychology Today — Trauma and PTSD Specialists
psychologytoday.com/us/therapists/trauma-and-ptsd The most comprehensive therapist directory in the US. Filter by "Trauma and PTSD" as a specialty, then look for therapists who mention "complex trauma," "CPTSD," or "childhood trauma" in their profiles.

The International Association of Trauma Professionals (IATP) iatp.org A trauma-specific therapist directory. Therapists listed here have completed additional training in trauma treatment.

EMDR International Association (EMDRIA)
emdria.org Directory of EMDR-trained therapists. EMDR is one of the most researched treatments for trauma and can be particularly effective for single-event trauma; for CPTSD, look for therapists who also have training in complex trauma or the "Recent Traumatic Episode Protocol" (R-TEP).

Somatic Experiencing International (SEI)
traumahealing.org Directory of Somatic Experiencing practitioners. For those interested in body-based approaches to trauma.

Internal Family Systems (IFS) Directory ifs-institute.com Directory of IFS-trained therapists. IFS has become increasingly recognised as effective for complex trauma and CPTSD.

Crisis Resources

If you are in crisis or need immediate support, please reach out. These resources are available 24/7 unless otherwise noted.

National Suicide Prevention Lifeline (US) 988 or 1-800-273-8255 Call or text 988. The Lifeline provides free, confidential support.

Crisis Text Line (US, Canada, UK, Ireland) Text HOME to 741741 (US) / 686868 (Canada) / 85258 (UK) / 50808 (Ireland) Free, 24/7 crisis support via text message.

National Domestic Violence Hotline (US) 1-800-799-7233 (TTY 1-800-787-3224) thehotline.org Call, chat, or text. Advocates are available 24/7 in over 200 languages.

RAINN (Rape, Abuse & Incest National Network) (US) 1-800-656-HOPE (4673) rainn.org Online chat available. The nation's largest anti-sexual violence organisation.

Samaritans (UK and Ireland) 116 123 (free from any phone) samaritans.org 24/7 emotional support for anyone in distress.

International Association for Suicide Prevention
iasp.info/resources/Crisis_Centres/ A directory of crisis centres organised by country.

A Note on Therapy Modalities

Different approaches work for different people. There is no single "right" therapy for CPTSD. Here is a brief overview of modalities commonly used:

EMDR (Eye Movement Desensitization and Reprocessing): Uses bilateral stimulation (typically eye movements) to help the brain reprocess traumatic memories. Evidence-supported for PTSD; increasingly used for CPTSD with modifications.

Somatic Experiencing: A body-focused approach developed by Peter Levine that works with the physical sensations of trauma rather than the narrative. Gentle, bottom-up processing.

Internal Family Systems (IFS): Views the mind as containing multiple "parts" (including the inner critic) and works to heal the relationship between parts and the core Self. Increasingly evidence-supported for complex trauma.

Cognitive Behavioural Therapy (CBT): Focuses on identifying and changing unhelpful thought patterns. Most effective for single-event PTSD; has limitations for CPTSD. Many trauma specialists now recommend trauma-informed CBT or CBT adapted for complex trauma.

Dialectical Behaviour Therapy (DBT): Originally developed for borderline personality disorder, DBT is also effective for CPTSD symptoms including emotional dysregulation and relationship difficulties. Focuses on distress tolerance, mindfulness, interpersonal effectiveness, and emotion regulation.

Cognitive Processing Therapy (CPT): A structured approach that helps patients challenge and modify unhelpful beliefs related to trauma. Evidence-supported for PTSD.

Final Note

These resources are here to support you, not to overwhelm you. You do not need to read every book, listen to every podcast, or try every therapy. Healing does not work by consuming more information. It works by taking small, consistent steps toward safety, self-understanding, and self-compassion.

Pick one book that feels right. Or one podcast episode. Or one therapist directory search. Start there. Let that lead you to the next thing.

The heavy little things get lighter. Not all at once, not in a straight line, but over time. The resources in this section have helped others along that path. May they help you find the next step on yours.

About the Author

Elena Voss, MSc, is a researcher and writer specialising in women's mental health and neurodivergence. She holds a Master's degree in Psychology and has spent over a decade studying the intersection of mood disorders, hormonal health, and women's lived experiences.

Her work focuses on translating clinical research into practical, actionable strategies that women can use in their daily lives. She is the author of multiple books on ADHD, autism, AuDHD, and bipolar disorder in women, all written under the guiding principle that knowledge is only useful if it can be applied.

Elena lives in the UK with her family and an ever-growing collection of notebooks. She is an advocate for neurodiversity awareness and

believes that every woman deserves access to tools that help her understand and work with her own brain.

Also by Elena Voss, MSc

ADHD in Women: The Late Diagnosis Guide

The AuDHD Woman: Understanding Autism and ADHD
as a Dual Reality

Anxious Woman's Compass: Navigating Anxiety with
Practical Tools

The Autism Toolkit for Women: Practical Strategies for
Navigating Daily Life, Work, and Relationships

The Weather Within: A Bipolar Guide for Women

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